ENGLISH FOR SPORTS MEDICINE PROFESSIONALS

Sports Medicine English

Describe the movement. Clarify the next step.



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Eight lessons. Eleven extended conversations.

INSIDE THE BOOK

192 vocabulary entries / 128 reusable phrases
220 extended-dialogue turns / 11 transfer exchanges
Word banks and explained answer keys

Upper-intermediate to advanced (B2-C1). For sports physicians, physiotherapists, athletic trainers, and rehabilitation professionals working within their authorized roles.

2026 edition

Eight scenarios. Eight skills.

Eight encounters practice mechanism-of-injury questions, examination explanations, rehabilitation read-backs, concussion communication, training-load comparisons, imaging limits, return criteria, and information sharing.

01 Clarifying an injury mechanism 4

A twist, not a collision | Elicit movement and symptom details without turning an athlete's description into an unconfirmed diagnosis.

02 Explaining an examination and inviting a pause 13

Active movement is not passive movement | Give clear examination explanations and check consent without converting cooperation into blanket permission.

03 Reading back a rehabilitation plan 22

The current plan is not the next progression | Clarify exercise-plan language, version changes, and review ownership without inventing treatment details.

04 Communicating concussion follow-up clearly 31

Back in class is not back in contact practice | Distinguish return-to-learn arrangements from return-to-sport authorization without minimizing symptoms.

05 Comparing training load without false equivalence 40

The same minutes, a different session | Compare activity records while separating duration, intensity, and the limits of monitoring data.

06 Explaining imaging findings in clinical context 49

An image is not a return-to-play decision | Explain the distinction between a structural report, symptoms, function, and a participation decision.

07 Discussing return-to-sport criteria under pressure 58

The final is a date, not a criterion | Communicate a conditional participation decision while acknowledging competitive pressure and uncertainty.

08 Sharing participation information without oversharing 67

The coach needs the limit, not the whole chart | Clarify the recipient, purpose, and authorized scope of a sports-medicine update.

Communication field notes 3

Three more workplace conversations 76

Answers and explanations 85

Fast-access phrase pages 104

Vocabulary index 106

Sources and publication notes 114

Participation is not the same as clearance.

Sports medicine conversations connect symptoms and performance with clinical assessment, rehabilitation, and pressure to compete. Accurate English names the activity, evidence, restriction, and responsible decision-maker without turning a hopeful timetable into permission.

Describe before labeling

Clarify contact, movement, timing, and symptoms before adopting the athlete's informal diagnosis. A pop, a twist, or a feeling of instability is information to assess, not a complete diagnosis.

"When you say it gave way, what actually happened during the movement?"

Separate instruction from progression

A current rehabilitation plan does not automatically authorize the next activity. Read back the actual instruction and clarify changes with the responsible clinician.

"This is the current plan; progression has not been approved in this update."

Make the activity specific

Training, practice, contact, competition, and performance are not interchangeable. A person may participate in one approved activity without being cleared for every sporting demand.

"The note allows the specified activity, not unrestricted competition."

Protect the athlete's voice

Performance pressure does not remove privacy, consent, or clinical responsibility. Explain what can be shared, with whom, and for what purpose under the actual rules.

"We can communicate authorized participation limits without forwarding the full clinical record."

PROFESSIONAL BOUNDARIES

All people, organizations, cases, figures, and documents in the exercises are fictional. This is professional English practice, not medical advice, clinical training, an exercise prescription, or clearance to play. Follow actual emergency procedures, qualified clinical assessment, current evidence, local law, and sport-specific rules. Do not diagnose an injury, calculate a dose, select treatment, or advance rehabilitation from an exercise. Concussion and return-to-sport decisions require the appropriate healthcare professional. Protect athlete information and clarify the authority of each role.

A twist, not a collision

A Read the situation

Elicit movement and symptom details without turning an athlete's description into an unconfirmed diagnosis.

Football player Luca reports left-knee discomfort after planting the left foot and turning during practice. No other player made contact. Luca says the knee gave way but cannot explain whether that meant pain, buckling, or a fall. Dr Arun is taking the history; no examination, scan result, or diagnosis is supplied. The task is accurate history language, not deciding whether the injury is safe to ignore.

PEOPLE / Dr Arun: Sports physician | Luca: Athlete

THE ATHLETE'S WORDS NEED ROOM

An everyday phrase such as gave way may have several meanings. Ask for the actual experience before substituting a clinical label.

Briefing checks

Circle one answer in each item. Use the briefing.

1. Which mechanism is reported?

A A confirmed ligament rupture
B Planting the left foot and turning without contact
C A direct collision to the right knee
D A fall from height

2. Which phrase needs clarification?

A No player contact
B Gave way
C Left foot
D During practice

3. What is not yet supplied?

A The practice setting
B A confirmed diagnosis
C A reported movement
D The side involved

Answers and explanations: page 85.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

mechanism of injury

Movement or event associated with the reported injury.

IN USE clarify the mechanism of injury

non-contact injury

Injury associated with an event without direct contact from another person or object.

IN USE describe a non-contact injury

pivot

Turn made around a planted or supporting foot.

IN USE describe a pivot

weight-bearing

Supporting body weight through a limb.

IN USE clarify weight-bearing ability

joint instability

Sense or finding that a joint does not maintain expected stability.

IN USE distinguish reported joint instability

buckling

Sudden giving way during support or movement.

IN USE describe an episode of buckling

locking

Reported or assessed inability to move a joint through an expected range.

IN USE clarify reported locking

swelling

Enlargement associated with fluid or tissue change.

IN USE describe the onset of swelling

effusion

Fluid accumulation within a joint or body space.

IN USE assess a joint effusion

ligament

Connective tissue linking bones across a joint.

IN USE explain ligament anatomy

tendon

Tissue connecting muscle to bone.

IN USE distinguish a tendon from a ligament

meniscus

Fibrocartilage structure in the knee contributing to load distribution and stability.

IN USE explain the meniscus

Precision check

A non-contact pivot describes a mechanism, not a confirmed ligament or meniscal injury. Gave way needs clarification and must not be recorded as an examination finding.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

sprain

Injury involving a ligament.

IN USE distinguish a sprain

strain

Injury involving muscle or tendon.

IN USE distinguish a strain

symptom onset

Time or sequence in which symptoms began.

IN USE establish symptom onset

aggravating factor

Activity or circumstance that worsens symptoms.

IN USE identify an aggravating factor

relieving factor

Activity or circumstance associated with reduced symptoms.

IN USE identify a relieving factor

prior injury

Earlier injury relevant to the current history.

IN USE clarify a prior injury

clarification request

Question that resolves an unclear meaning or instruction.

IN USE make a targeted clarification request

source attribution

Identification of where information came from.

IN USE preserve source attribution

working assumption

Provisional premise used while information is incomplete.

IN USE challenge a working assumption

scope boundary

Limit of the work or authority being discussed.

IN USE state the scope boundary

action owner

Named person responsible for a specified next step.

IN USE confirm the action owner

check-back

Restatement used to verify that a message was understood.

IN USE complete a check-back

Precision check

Symptoms, assessed function, and structural diagnosis answer different questions. Missing examination and imaging information must not become a normal result or reassurance.

Phrases you can actually use

C Notice the language / 1 of 2

LOCATE THE EVENT

"Which foot was on the ground when you turned?"

CLARIFY CONTACT

"Did anyone or anything make contact with you?"

UNPACK THE PHRASE

"When you say it gave way, what did you feel or see?"

SEPARATE SEQUENCE

"Did the discomfort begin before, during, or after the turn?"

ASK ABOUT FUNCTION

"What happened when you tried to support weight afterward?"

PRESERVE UNCERTAINTY

"That describes the event; it does not confirm the injured structure."

CHECK THE SIDE

"I am confirming that you mean the left knee."

AVOID A SHORTCUT

"We should not replace your description with a diagnosis yet."

Felt versus found

Felt attributes an experience to the athlete; found implies an assessment finding. Keep the source explicit.

During versus after

These words locate onset in the sequence and should not be used interchangeably.

No contact

No contact describes the mechanism and does not mean no injury or no need for assessment.

Phrases you can actually use

C Notice the language / 2 of 2

CLARIFY THE QUESTION

"Are you asking about the finding or the next step?"

INVITE CORRECTION

"Please correct me if I have misunderstood that detail."

ATTRIBUTE INFORMATION

"That is what the current record says; it has not been independently confirmed."

SEPARATE POSSIBILITY

"That is a possibility, not an agreed outcome."

CHECK SCOPE

"That decision sits outside my authority."

PIN DOWN RESPONSIBILITY

"Who is responsible for that specific next step?"

REPAIR MISUNDERSTANDING

"Let me restate the distinction more clearly."

CLOSE AN EXCHANGE

"We agree on the action, the owner, and the point for review."

Reported versus verified

Reported attributes information to a source; verified indicates it has been checked. Do not use the words interchangeably.

Could versus will

Could introduces a possibility. Will commits to an action or predicts an outcome; check whether that certainty is justified.

By versus at

By gives a latest time; at identifies a scheduled moment. State the time zone when people work across locations.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which opening question resolves the key ambiguity?

A So your examination was normal?
B What happened when you say the knee gave way?
C Which ligament definitely ruptured?
D Why did you keep playing after a diagnosis?

2. Which summary preserves the reported mechanism?

A Confirmed meniscal tear on imaging
B No injury because there was no contact
C Left foot planted; turn without player contact
D Right knee struck by another player

3. Which distinction is anatomically appropriate?

A A sprain and strain always name the same tissue
B A tendon connects two bones across every joint
C A meniscus is a muscle
D A sprain involves a ligament; a strain involves muscle or tendon

4. Which claim exceeds the case?

A The left foot was planted
B The ligament is definitely torn
C The athlete reports discomfort
D The event occurred at practice

Answers and explanations: page 85.

A twist, not a collision

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

strain / non-contact injury / pivot / ligament / aggravating factor / swelling / prior injury / buckling / mechanism of injury / weight-bearing

01 **Dr Arun:** Tell me what happened just before the knee discomfort started, including where your foot was on the ground.

02 **Luca:** My left foot was planted, and I turned. The [1] _____ was not a tackle; nobody made contact with me.

03 **Dr Arun:** I will record that distinction. When you said it gave way, what actually happened during the movement?

04 **Luca:** I am not sure whether [2] _____ is the right word. I felt something unexpected and stopped, but I did not fall.

05 **Dr Arun:** We can keep your own description while clarifying it. Did the discomfort begin during the turn or afterward?

06 **Luca:** During the [3] _____, as far as I remember. I cannot separate the exact moment any more precisely than that.

07 **Dr Arun:** That uncertainty is useful to preserve. What happened when you tried to put weight through the leg afterward?

08 **Luca:** I can describe my [4] _____ experience, but I do not want my answer treated as proof of what structure is injured.

09 **Dr Arun:** It will be part of the history, not a substitute for the actual assessment. Did you notice any change in size?

10 **Luca:** I noticed some [5] _____ later, but I cannot say whether it began immediately because I was concentrating on the discomfort.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 10.

11 **Dr Arun:** We should distinguish that report from what is found on examination. Has anything similar happened before?

12 **Luca:** There was a [6] _____ to this knee, although I do not have the old records with me today.

13 **Dr Arun:** We can clarify those separately. What movement seems to make the current discomfort more noticeable?

14 **Luca:** Turning feels like an [7] _____, but I have not been trying movements just to test the knee myself.

15 **Dr Arun:** That account helps. Have you been told a diagnosis, or are the terms you have heard guesses from teammates?

16 **Luca:** They mentioned a [8] _____ tear and a meniscus problem, but those were suggestions rather than a clinician's findings.

17 **Dr Arun:** Then we will not copy them into the record as confirmed. Different structures and symptoms need proper assessment.

18 **Luca:** I also mixed up sprain and [9] _____. I understand now that the words do not simply mean mild and severe versions of the same injury.

19 **Dr Arun:** Exactly. The next clinical assessment uses the actual history and findings; this description does not settle the diagnosis.

20 **Luca:** Then my report is a [10] _____ event with left-knee discomfort and some uncertain details, not a confirmed tear.

ANSWERS / EXPLANATIONS: PAGE 86

1. mechanism of injury | 2. buckling | 3. pivot | 4. weight-bearing | 5. swelling | 6. prior injury | 7. aggravating factor | 8. ligament | 9. strain | 10. non-contact injury

The side was entered incorrectly

F Say it

An athlete clarifies that the right ankle was involved, not the left. No diagnosis is supplied. The clinician follows the actual correction process and retains the source of the clarification.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

right / source / left / unconfirmed

Athlete: "The affected ankle was the [1] _____ one."

Clinician: "The earlier [2] _____ entry needs correction."

Athlete: "The diagnosis remains [3] _____."

Clinician: "I will preserve the correction's [4] _____."

ANSWERS / EXPLANATIONS: PAGE 86

1. right | 2. left | 3. unconfirmed | 4. source

Rehearse with a fixed script

1. Read the corrected history and stress left, non-contact, and reported.

2. Change roles and clarify gave way without inserting a structural diagnosis.

3. Complete the four-line follow-up with the supplied choices. Check the explanations, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Active movement is not passive movement

A Read the situation

Give clear examination explanations and check consent without converting cooperation into blanket permission.

Physiotherapist Elena explains a planned shoulder examination to athlete Mo. She distinguishes movement Mo performs from movement guided by the examiner and explains that Mo can ask to pause. No maneuver, measured angle, diagnosis, or exercise prescription is supplied. Mo initially thinks range of motion means strength. The conversation practices explanation and consent within the actual clinical process.

PEOPLE / Elena: Physiotherapist | Mo: Athlete

CONSENT STAYS ACTIVE

Explain a step before it happens. Agreeing to an appointment does not remove the person's ability to ask questions, decline a step, or request a pause.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What does Mo initially confuse?

A Movement range and strength
B Appointment time and date
C Shoulder and ankle
D Consent and an imaging result

2. Which action remains available to Mo?

A Ask to pause
B Authorize every future procedure automatically
C Diagnose the shoulder from the dialogue
D Choose an invented maneuver

3. What is not supplied?

A A planned explanation
B A patient question
C The clinician's role
D A measured angle

Answers and explanations: page 87.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

range of motion

Extent of movement available at a joint.

IN USE assess range of motion

active movement

Movement performed by the person using their own muscles.

IN USE explain active movement

passive movement

Movement guided by another person or an external force.

IN USE explain passive movement

resisted movement

Movement or effort made against an applied resistance.

IN USE describe resisted movement

flexion

Bending movement that generally decreases a joint angle.

IN USE explain shoulder flexion in context

extension

Movement generally increasing a joint angle or moving from flexion.

IN USE describe extension

abduction

Movement away from the relevant body midline.

IN USE explain abduction

adduction

Movement toward the relevant body midline.

IN USE explain adduction

internal rotation

Rotation toward the relevant anatomical midline.

IN USE describe internal rotation

external rotation

Rotation away from the relevant anatomical midline.

IN USE describe external rotation

palpation

Examination by touch using an appropriate clinical technique.

IN USE explain palpation before contact

tenderness

Pain or discomfort elicited by examination pressure.

IN USE document assessed tenderness

Precision check

Range of motion describes movement extent, not muscle strength. Active and passive describe how movement occurs, not whether the athlete is cooperative.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

muscle strength

Capacity to produce force in the relevant assessment.

IN USE assess muscle strength

functional task

Activity used to examine a relevant everyday or sporting demand.

IN USE clarify a functional task

baseline measurement

Initial measurement used for later comparison.

IN USE record a baseline measurement

examination consent

Agreement to an explained examination within the actual consent process.

IN USE confirm examination consent

chaperone

Person present in an examination under the relevant policy and preference.

IN USE offer a chaperone appropriately

stop signal

Agreed way to request a pause or stop.

IN USE establish a stop signal

clarification request

Question that resolves an unclear meaning or instruction.

IN USE make a targeted clarification request

source attribution

Identification of where information came from.

IN USE preserve source attribution

working assumption

Provisional premise used while information is incomplete.

IN USE challenge a working assumption

scope boundary

Limit of the work or authority being discussed.

IN USE state the scope boundary

action owner

Named person responsible for a specified next step.

IN USE confirm the action owner

check-back

Restatement used to verify that a message was understood.

IN USE complete a check-back

Precision check

This script explains terms, not examination technique. Actual steps, consent, chaperone arrangements, and clinical responses follow the relevant professional process.

Phrases you can actually use

C Notice the language / 1 of 2

EXPLAIN THE PURPOSE

"This part looks at how the shoulder moves, not only how strong it is."

DISTINGUISH THE ACTION

"Active means you move it yourself; passive means movement is guided."

ASK BEFORE CONTACT

"I will explain the next step before any contact."

INVITE A PAUSE

"Please tell me if you want to pause or stop."

CHECK MEANING

"How would you describe that distinction in your own words?"

AVOID A LABEL

"Passive does not mean you are being unhelpful."

CLARIFY THE MEASURE

"A range measurement is not a complete diagnosis."

KEEP CONSENT SPECIFIC

"We can discuss this step separately before proceeding."

Can you versus you must

Can you may invite an action, but the surrounding explanation must make the person's ability to pause or decline clear.

Active/passive

These are movement terms in this context, not character judgments or grammatical voice labels.

Before

Before marks the timing of explanation and permission relative to contact or the next examination step.

Phrases you can actually use

C Notice the language / 2 of 2

CLARIFY THE QUESTION

"Are you asking about the finding or the next step?"

INVITE CORRECTION

"Please correct me if I have misunderstood that detail."

ATTRIBUTE INFORMATION

"That is what the current record says; it has not been independently confirmed."

SEPARATE POSSIBILITY

"That is a possibility, not an agreed outcome."

CHECK SCOPE

"That decision sits outside my authority."

PIN DOWN RESPONSIBILITY

"Who is responsible for that specific next step?"

REPAIR MISUNDERSTANDING

"Let me restate the distinction more clearly."

CLOSE AN EXCHANGE

"We agree on the action, the owner, and the point for review."

Reported versus verified

Reported attributes information to a source; verified indicates it has been checked. Do not use the words interchangeably.

Could versus will

Could introduces a possibility. Will commits to an action or predicts an outcome; check whether that certainty is justified.

By versus at

By gives a latest time; at identifies a scheduled moment. State the time zone when people work across locations.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which explanation distinguishes active and passive correctly?

A You move it yourself versus movement guided by another
B Helpful versus unhelpful behavior
C Strong versus weak tissue
D Safe versus dangerous diagnosis

2. Which instruction supports ongoing consent?

A You attended, so every step is agreed
B Remain silent even if unclear
C Questions can only be asked afterward
D Tell me if you want to pause before we continue

3. Which pair should not be treated as identical?

A Explanation and explanation
B Range of motion and muscle strength
C Left and left
D Appointment and scheduled visit

4. Which claim would invent a finding?

A No angle is supplied here
B Your range is exactly normal
C This step concerns movement
D You can ask for a pause

Answers and explanations: page 87.

Active movement is not passive movement

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

baseline measurement / palpation / active movement / range of motion / functional task / passive movement / chaperone / stop signal / muscle strength / examination consent

01 **Elena:** Before the examination, I would like to explain what we are looking at and how you can ask to pause.

02 **Mo:** Thank you. I thought [1] _____ was another way of saying how strong my shoulder is.

03 **Elena:** They are different questions. Movement extent and force production can both be relevant, but one does not replace the other.

04 **Mo:** So a [2] _____ gives you a starting point rather than a complete diagnosis from one number.

05 **Elena:** Exactly. I will also explain whether a movement is one you perform or one that is guided during examination.

06 **Mo:** Would [3] _____ mean that I move the shoulder myself, using my own muscles?

07 **Elena:** Yes. The term describes how the movement happens, not whether you are trying hard enough to please the examiner.

08 **Mo:** Then [4] _____ would mean movement is guided, not that I am being unhelpful or refusing to participate.

09 **Elena:** Correct. I will explain each step before we do it; I am not asking you to try unfamiliar examination techniques yourself.

10 **Mo:** I appreciate that. I would like an explanation before [5] _____ or any other contact, rather than being surprised halfway through.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 19.

11 **Elena:** We can do that. Please also tell me if something is unclear or you want to pause before the next step.

12 **Mo:** Could we agree on a [6] _____ now? Sometimes I hesitate when a clinician seems busy or focused.

13 **Elena:** Yes, and you can say stop or ask a question directly. Coming to the appointment does not remove that choice.

14 **Mo:** That makes [7] _____ feel specific to what we discuss, rather than permission for every possible procedure.

15 **Elena:** Precisely. We can also discuss the relevant chaperone arrangements and your preferences under the service's actual policy.

16 **Mo:** I would like to understand when a [8] _____ is offered and how that works before deciding about the appointment arrangements.

17 **Elena:** We will explain it. We should also keep the words movement, strength, and function separate in the summary.

18 **Mo:** A [9] _____ would relate to an actual activity, rather than only one isolated angle or force measurement.

19 **Elena:** Exactly. The real assessment determines which information is appropriate and what it means in your situation.

20 **Mo:** Then [10] _____ is one part of the discussion, not a word I should use for every aspect of the shoulder examination.

ANSWERS / EXPLANATIONS: PAGE 88

1. range of motion | 2. baseline measurement | 3. active movement | 4. passive movement | 5. palpation | 6. stop signal | 7. examination consent | 8. chaperone | 9. functional task | 10. muscle strength

The request was to pause, not to finish

F Say it

During an examination explanation, an athlete asks to pause for clarification. No measurement has yet been taken and no finding is established.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

explanation / finding / pause / taken

Athlete: "I would like to [1] _____ before the next step."

Clinician: "Let us clarify the [2] _____ first."

Athlete: "No measurement has been [3] _____."

Clinician: "There is no established examination [4] _____."

ANSWERS / EXPLANATIONS: PAGE 88

1. pause | 2. explanation | 3. taken | 4. finding

Rehearse with a fixed script

1. Read the corrected explanation and contrast active, passive, range, and strength.

2. Swap roles and practice asking for a pause before an unexplained step; do not perform examination maneuvers.

3. Complete the four-line follow-up with the supplied choices. Check the explanations, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

The current plan is not the next progression

A Read the situation

Clarify exercise-plan language, version changes, and review ownership without inventing treatment details.

Athlete Tessa has two rehabilitation documents. The current signed plan specifies supervised rehabilitation and a review before any progression. An older handout uses broader wording about practice. Physiotherapist Omar confirms the current version and the responsible clinician. No exercise name, dosage, resistance, or progression criterion is supplied in this exercise.

PEOPLE / Tessa: Athlete | Omar: Physiotherapist

INSTRUCTIONS MUST SURVIVE THE JOURNEY HOME

Read-back checks whether the explanation was clear. It should not become a test that rewards guessing an unsupplied exercise or dosage.

Briefing checks

Circle one answer in each item. Use the briefing.

1. Which document governs the supplied communication?

A A teammate's routine
B A guessed exercise schedule
C The current signed rehabilitation plan
D The older broad handout

2. What must happen before progression?

A The stated clinical review
B A competition date arriving
C Buying new equipment
D Completing this English exercise

3. Which detail is absent?

A A current plan
B An older handout
C The need for review
D Exercise dosage

Answers and explanations: page 89.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

rehabilitation plan

Agreed clinical plan for recovery of relevant function.

IN USE clarify the rehabilitation plan

exercise prescription

Authorized specification of exercise within a clinical plan.

IN USE verify an exercise prescription

repetition

One complete performance of a specified movement.

IN USE distinguish a repetition

set

Group of repetitions performed under a specified plan.

IN USE distinguish a set

resistance

Load opposing a movement or effort.

IN USE specify the actual resistance

frequency

How often an activity is prescribed or performed.

IN USE clarify exercise frequency

duration

Length of time an activity lasts.

IN USE distinguish duration from frequency

progression

Planned movement to a different activity or demand under relevant criteria.

IN USE authorize a progression

regression

Adjustment to a less demanding activity within a clinical plan.

IN USE clarify a regression

supervised session

Session conducted with the specified professional oversight.

IN USE confirm a supervised session

home program

Prescribed activities intended for the home setting.

IN USE review a home program

load tolerance

Response to a specified amount or type of demand.

IN USE assess load tolerance

Precision check

Sets, repetitions, resistance, frequency, and duration are distinct details. None can be inferred from a generic instruction to do more, and none is supplied here.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

exercise adherence

Extent to which an agreed exercise plan is followed.

IN USE discuss exercise adherence accurately

symptom response

Change in symptoms during or after an activity.

IN USE report the symptom response

review criterion

Condition or information used at a clinical review.

IN USE clarify a review criterion

prescribing clinician

Professional responsible for the relevant prescribed plan.

IN USE identify the prescribing clinician

superseded instruction

Earlier direction replaced by a current one.

IN USE identify a superseded instruction

plan version

Identifiable issue of a plan distinguished by date or revision.

IN USE confirm the plan version

clarification request

Question that resolves an unclear meaning or instruction.

IN USE make a targeted clarification request

source attribution

Identification of where information came from.

IN USE preserve source attribution

working assumption

Provisional premise used while information is incomplete.

IN USE challenge a working assumption

scope boundary

Limit of the work or authority being discussed.

IN USE state the scope boundary

action owner

Named person responsible for a specified next step.

IN USE confirm the action owner

check-back

Restatement used to verify that a message was understood.

IN USE complete a check-back

Precision check

The current plan requires review before progression. A language read-back confirms understanding; it does not authorize exercise or advancement.

Phrases you can actually use

C Notice the language / 1 of 2

CHECK THE VERSION

"Which dated plan are we reading together?"

SEPARATE INSTRUCTIONS

"That wording belongs to the older handout, not the current plan."

CLARIFY THE UNIT

"Do you mean sets, repetitions, or the length of the session?"

READ BACK

"My understanding is supervised rehabilitation, then review before progression."

IDENTIFY AUTHORITY

"Which clinician can clarify or change this instruction?"

AVOID GUESSING

"I cannot supply a resistance value that is not written here."

REPORT RESPONSE

"I will describe what happened during and after the specified activity."

CONFIRM THE LIMIT

"Completing this conversation does not change the clinical plan."

Before progression

Before makes the review a prior step rather than a discussion that can be postponed until after advancement.

More

More is incomplete without the relevant quantity or dimension: repetitions, load, frequency, and duration differ.

Current versus older

The date and authority of the plan matter; a familiar older handout is not automatically the governing instruction.

Phrases you can actually use

C Notice the language / 2 of 2

CLARIFY THE QUESTION

"Are you asking about the finding or the next step?"

INVITE CORRECTION

"Please correct me if I have misunderstood that detail."

ATTRIBUTE INFORMATION

"That is what the current record says; it has not been independently confirmed."

SEPARATE POSSIBILITY

"That is a possibility, not an agreed outcome."

CHECK SCOPE

"That decision sits outside my authority."

PIN DOWN RESPONSIBILITY

"Who is responsible for that specific next step?"

REPAIR MISUNDERSTANDING

"Let me restate the distinction more clearly."

CLOSE AN EXCHANGE

"We agree on the action, the owner, and the point for review."

Reported versus verified

Reported attributes information to a source; verified indicates it has been checked. Do not use the words interchangeably.

Could versus will

Could introduces a possibility. Will commits to an action or predicts an outcome; check whether that certainty is justified.

By versus at

By gives a latest time; at identifies a scheduled moment. State the time zone when people work across locations.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which read-back matches the plan?

A Unrestricted practice now
B Progress whenever the calendar changes
C Copy the older handout because it is shorter
D Supervised rehabilitation, with review before progression

2. Why is do more an inadequate instruction?

A It leaves the dimension of change unclear
B It always means double the resistance
C It always means add another day
D It automatically replaces supervision

3. Which action handles the conflicting documents appropriately?

A Verify the current version with the responsible clinician
B Combine both into a new plan
C Choose whichever sounds easier
D Treat the older date as stronger authority

4. Which claim exceeds this exercise?

A The athlete should use a particular resistance
B There are two documents
C The current plan requires review
D The older wording is broader

Answers and explanations: page 89.

The current plan is not the next progression

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

plan version / symptom response / prescribing clinician / frequency / superseded instruction / supervised session / review criterion / repetition / rehabilitation plan / Progression

01 **Tessa:** I have two documents, and the older one makes it sound as though I can return to practice now.

02 **Omar:** Let us verify the [1] _____ before combining instructions that were written at different stages of your care.

03 **Tessa:** This signed page is the current one. It says supervised rehabilitation and review before any progression.

04 **Omar:** Then that is the [2] _____ we need to explain, while the older wording is clarified through the appropriate process.

05 **Tessa:** I had also written do more in my own notes, but now I cannot remember what more referred to.

06 **Omar:** That does not identify [3] _____, duration, resistance, or repetitions. We should not guess which detail was intended.

07 **Tessa:** I sometimes confuse a set with a repetition. I want to understand the words before reading the actual instructions.

08 **Omar:** A [4] _____ is one complete performance of a movement; a set is a group of repetitions in the plan.

09 **Tessa:** Understanding those terms does not give me permission to add either one when I feel confident, does it?

10 **Omar:** Correct. [5] _____ follows the clinical decision and criteria, not simply your understanding of the terms or the competition calendar.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 28.

11 **Tessa:** The current page also says supervised. I should not turn that into an unsupervised home program myself.

12 **Omar:** Exactly. A [6] _____ is a different arrangement, and any change needs the appropriate clarification and authority.

13 **Tessa:** If I notice a change during the activity, what language helps me report it accurately to the clinician?

14 **Omar:** Describe the actual [7] _____, including when it occurred, while following your real clinical instructions and contact pathway.

15 **Tessa:** I should not decide that discomfort means the plan is definitely working, or that any change proves a new injury.

16 **Omar:** Right. The [8] _____ needs the actual information rather than a conclusion added to sound certain.

17 **Tessa:** Could we mark which older wording is no longer the instruction so I do not repeat it to my coach?

18 **Omar:** We should identify the [9] _____ through the approved process and ensure the corrected message reaches the relevant people.

19 **Tessa:** My read-back is supervised rehabilitation, then the stated review before progression, with no invented numbers or exercises.

20 **Omar:** That preserves the [10] _____ and the clinician's authority. I will clarify the missing detail before we treat any progression as agreed.

ANSWERS / EXPLANATIONS: PAGE 90

1. plan version | 2. rehabilitation plan | 3. frequency | 4. repetition | 5. Progression | 6. supervised session | 7. symptom response | 8. prescribing clinician | 9. superseded instruction | 10. review criterion

More often is not more resistance

F Say it

An athlete asks whether more often and greater resistance mean the same thing. No change to the actual rehabilitation plan has been prescribed.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

prescribed / load / clarification / frequency

Athlete: "More often refers to [1] _____."

Clinician: "Greater resistance changes the opposing [2] _____."

Athlete: "No plan change has been [3] _____."

Clinician: "The actual instruction needs appropriate [4] _____."

ANSWERS / EXPLANATIONS: PAGE 90

1. frequency | 2. load | 3. prescribed | 4. clarification

Rehearse with a fixed script

1. Read the corrected exchange and distinguish sets, repetitions, frequency, and duration.

2. Swap roles and read back the current plan without adding an exercise, dose, or progression rule.

3. Complete the four-line follow-up with the supplied choices. Check the explanations, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Back in class is not back in contact practice

A Read the situation

Distinguish return-to-learn arrangements from return-to-sport authorization without minimizing symptoms.

Student athlete Avery is following an actual clinician-led concussion process. The current school note permits specified classroom adjustments; it does not authorize contact practice or competition. Coach Morgan asks whether attending class means Avery is cleared to play. Athletic trainer Sal explains the difference and identifies the responsible healthcare professional. No symptom score, examination result, or progression step is supplied.

PEOPLE / Morgan: Coach | Sal: Athletic trainer

A POSITIVE STEP CAN STILL HAVE LIMITS

Acknowledge progress without making it an all-clear. Coaches need precise participation information, not a reassuring phrase that erases the stated restrictions.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What does the current note address?

A Competition clearance
B A complete symptom assessment
C Specified classroom adjustments
D Unrestricted contact practice

2. Who must handle the clinical authorization question?

A A teammate
B The English exercise
C The responsible healthcare professional
D The coach alone

3. Which inference is unsupported?

A The process is clinician-led
B Attending class means cleared for contact play
C A school note exists
D The coach has a question

Answers and explanations: page 91.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

concussion

Brain injury requiring appropriate clinical assessment and management.

IN USE communicate a concussion concern

return to learn

Process of resuming learning with appropriate supports after concussion.

IN USE clarify return-to-learn arrangements

return to sport

Clinically guided process of resuming sporting activity.

IN USE clarify return-to-sport status

cognitive exertion

Mental effort associated with learning or other thinking tasks.

IN USE discuss cognitive exertion

physical exertion

Bodily effort associated with activity.

IN USE distinguish physical exertion

symptom inventory

Structured record of reported symptoms.

IN USE review a symptom inventory

headache

Pain experienced in the head.

IN USE report a headache accurately

dizziness

Reported sense of altered balance or spatial orientation.

IN USE describe dizziness

photophobia

Sensitivity or discomfort associated with light.

IN USE explain photophobia

concentration difficulty

Trouble sustaining or directing attention.

IN USE describe concentration difficulty

academic adjustment

Temporary change supporting participation in learning.

IN USE confirm an academic adjustment

contact practice

Practice involving the relevant sport's physical contact demands.

IN USE distinguish contact practice

Precision check

A return-to-learn note and sport clearance answer different questions. Classroom attendance does not establish readiness for contact practice or competition.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

non-contact activity

Activity without the specified contact demands.

IN USE specify non-contact activity

medical clearance

Authorization by the appropriate healthcare professional for a defined activity.

IN USE verify medical clearance

staged progression

Stepwise advancement under the applicable clinical process.

IN USE follow the authorized staged progression

symptom recurrence

Return of symptoms after improvement or absence.

IN USE report symptom recurrence

clinical reassessment

Further evaluation by the appropriate healthcare professional.

IN USE arrange clinical reassessment

participation status

Current authorized level and conditions of involvement.

IN USE communicate participation status

clarification request

Question that resolves an unclear meaning or instruction.

IN USE make a targeted clarification request

source attribution

Identification of where information came from.

IN USE preserve source attribution

working assumption

Provisional premise used while information is incomplete.

IN USE challenge a working assumption

scope boundary

Limit of the work or authority being discussed.

IN USE state the scope boundary

action owner

Named person responsible for a specified next step.

IN USE confirm the action owner

check-back

Restatement used to verify that a message was understood.

IN USE complete a check-back

Precision check

No symptom report or score is supplied here, and a single reassuring statement cannot replace the actual clinician-led process. Follow real emergency and concussion procedures for actual concerns.

Phrases you can actually use

C Notice the language / 1 of 2

SEPARATE THE PERMISSIONS

"The school note addresses classroom arrangements, not contact practice."

SPECIFY THE ACTIVITY

"Which exact activity are you asking about?"

AVOID ALL-CLEAR

"Progress in one setting is not unrestricted clearance."

IDENTIFY THE DECISION-MAKER

"The responsible healthcare professional must clarify that authorization."

PRESERVE THE NOTE

"Let us use the current written participation information."

REJECT CALENDAR PRESSURE

"The match date does not replace the clinical process."

CLARIFY A REPORT

"No symptom score or examination finding is supplied in this update."

CONFIRM RECEIPT

"Please read back the participation limit your team has received."

Back at school

This phrase identifies attendance, not the level of learning demand or permission for sport.

Cleared for

Cleared for needs a specific activity and scope; the word cleared alone invites an overbroad interpretation.

Yet

Not authorized yet describes the present status without promising when authorization will occur.

Phrases you can actually use

C Notice the language / 2 of 2

CLARIFY THE QUESTION

"Are you asking about the finding or the next step?"

INVITE CORRECTION

"Please correct me if I have misunderstood that detail."

ATTRIBUTE INFORMATION

"That is what the current record says; it has not been independently confirmed."

SEPARATE POSSIBILITY

"That is a possibility, not an agreed outcome."

CHECK SCOPE

"That decision sits outside my authority."

PIN DOWN RESPONSIBILITY

"Who is responsible for that specific next step?"

REPAIR MISUNDERSTANDING

"Let me restate the distinction more clearly."

CLOSE AN EXCHANGE

"We agree on the action, the owner, and the point for review."

Reported versus verified

Reported attributes information to a source; verified indicates it has been checked. Do not use the words interchangeably.

Could versus will

Could introduces a possibility. Will commits to an action or predicts an outcome; check whether that certainty is justified.

By versus at

By gives a latest time; at identifies a scheduled moment. State the time zone when people work across locations.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which reply corrects the coach's inference?

A Attending class always means fit for a match
B A quiet morning proves recovery
C The calendar supplies medical clearance
D Classroom arrangements do not authorize contact practice

2. What detail should accompany cleared?

A The athlete's jersey number
B The public's expectation
C The activity and conditions authorized
D The team's ranking

3. Which wording preserves the evidence limit?

A The examination is normal
B There is no risk of recurrence
C No symptom score is supplied in this update
D All symptoms have resolved

4. What does the match date establish?

A A normal assessment
B A scheduling pressure, not clinical authorization
C Automatic permission for contact
D Completion of every progression stage

Answers and explanations: page 91.

Back in class is not back in contact practice

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

symptom inventory / staged progression / Contact practice / Physical exertion / return to learn / clinical reassessment / participation status / medical clearance / Cognitive exertion / Return to sport

01 **Morgan:** Avery is back in class this week. Can I take that as permission to include them in contact practice?

02 **Sal:** No. The note addresses [1] _____ arrangements, which are different from permission to resume the sport's contact demands.

03 **Morgan:** I do not want to ignore progress, but the phrase back at school sounded like a general all-clear.

04 **Sal:** We can acknowledge progress while keeping the [2] _____ precise. The supplied note permits specified classroom adjustments, not contact practice.

05 **Morgan:** Would a shorter practice count as the same kind of adjustment, just with less time on the field?

06 **Sal:** That would be a new activity decision. [3] _____ and the demands of contact sport need their actual clinical authorization.

07 **Morgan:** Then a school arrangement involving reduced concentration demands is not a substitute for a sports decision.

08 **Sal:** Correct. [4] _____ is part of a different context, although the clinical team coordinates the overall recovery process.

09 **Morgan:** Who should answer the specific question about practice if the classroom note is not meant to answer it?

10 **Sal:** The responsible healthcare professional should clarify [5] _____ through the actual process, with the permitted activity and conditions made explicit.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 37.

11 **Morgan:** I have not seen a symptom score or an examination report. I should not tell the team they were normal.

12 **Sal:** Exactly. A [6] _____ is not supplied in this update, so we cannot create a reassuring result from its absence.

13 **Morgan:** The match is approaching, and people will ask when the athlete can return. I cannot promise a date from this note.

14 **Sal:** That is right. The actual [7] _____ is not replaced by the team calendar or a confident-sounding prediction.

15 **Morgan:** Could I at least say that non-contact activity and contact practice are separate categories when I explain the question?

16 **Sal:** Yes, but neither is authorized by this school note. [8] _____ has a particular meaning and needs its own decision in the clinical plan.

17 **Morgan:** If there is a new concern, we should use the actual clinical route rather than wait for an informal team discussion.

18 **Sal:** Correct. Any need for [9] _____ follows our concussion and emergency procedures. Please use the designated clinical contact, not an informal team discussion.

19 **Morgan:** My read-back is classroom adjustments only in this note, with the sport participation question still requiring clarification.

20 **Sal:** That is accurate. [10] _____ remains a separate authorization question; we will communicate the actual decision when it is made.

ANSWERS / EXPLANATIONS: PAGE 92

1. return to learn | 2. participation status | 3. Physical exertion | 4. Cognitive exertion | 5. medical clearance | 6. symptom inventory | 7. staged progression | 8. Contact practice | 9. clinical reassessment | 10. Return to sport

The message omitted the activity

F Say it

A team message says cleared, but the actual current note authorizes only a specified non-contact activity. It does not authorize competition.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

scope / non-contact / information / competition

Clinician: "The authorization is for the specified [1] _____ activity."

Coach: "It does not authorize [2] _____."

Clinician: "The shorter message lost the [3] _____."

Coach: "I will send the accurate participation [4] _____."

ANSWERS / EXPLANATIONS: PAGE 92

1. non-contact | 2. competition | 3. scope | 4. information

Rehearse with a fixed script

1. Read the corrected conversation and distinguish school attendance, contact practice, and clearance.

2. Swap roles and answer the coach's question without promising a date or inventing a normal assessment.

3. Complete the four-line follow-up with the supplied choices. Check the explanations, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

The same minutes, a different session

A Read the situation

Compare activity records while separating duration, intensity, and the limits of monitoring data.

Athlete Dani and sports clinician Priya compare two fictional training logs. Both sessions lasted 30 minutes. One was recorded as easy cycling; the other as repeated hard intervals. The logs use a stated perceived-effort scale, but no individualized training recommendation is supplied. Dani calls the sessions the same load because their durations match. Priya clarifies what the records do and do not establish.

PEOPLE / Dani: Athlete | Priya: Sports clinician

A NUMBER NEEDS ITS DEFINITION

Wearable metrics and training jargon can create apparent precision. Clarify what the number measures before comparing it or assigning a clinical meaning.

Briefing checks

Circle one answer in each item. Use the briefing.

1. Which detail is the same in both records?

A A verified clinical effect
B Duration of 30 minutes
C Intensity description
D Type of session

2. What distinguishes the sessions?

A Different prescribed medicines
B A guaranteed recovery difference
C Easy cycling versus repeated hard intervals
D Different documented injuries

3. What is not provided by this comparison?

A A shared duration
B Different effort descriptions
C An individualized progression recommendation
D Two activity records

Answers and explanations: page 93.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

training load

Amount and character of demand imposed by training.

IN USE describe training load

external load

Work performed, such as distance or power, measured outside the person's response.

IN USE quantify external load

internal load

Person's physiological or perceived response to the work.

IN USE describe internal load

training volume

Total amount of training under the stated measure.

IN USE compare training volume

intensity

Degree of effort or demand relative to the stated context.

IN USE distinguish training intensity

session RPE

Session rating of perceived exertion under a defined scale and method.

IN USE record session RPE

perceived exertion

Person's reported sense of effort.

IN USE clarify perceived exertion

interval training

Training alternating periods of specified effort and recovery.

IN USE describe interval training

steady-state activity

Activity maintained at a relatively consistent demand.

IN USE compare steady-state activity

recovery interval

Period between efforts within a training session.

IN USE specify the recovery interval

work-to-rest ratio

Relationship between effort and recovery periods.

IN USE clarify the work-to-rest ratio

weekly mileage

Distance covered in the stated week.

IN USE compare weekly mileage

Precision check

Equal minutes do not establish equal intensity or overall demand. A perceived-effort score and a device estimate are different kinds of information, not automatic diagnoses or permission to progress.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

acute workload

Recent training demand over the defined short period.

IN USE define the acute workload window

chronic workload

Training demand over the defined longer period.

IN USE define the chronic workload window

fatigue

Experience or state of reduced capacity associated with effort or other factors.

IN USE report fatigue in context

recovery status

Current condition relevant to recovery, assessed through appropriate information.

IN USE discuss recovery status

wearable metric

Measurement or estimate reported by a wearable device.

IN USE interpret a wearable metric

monitoring limitation

Constraint on what a measurement or record can establish.

IN USE state a monitoring limitation

clarification request

Question that resolves an unclear meaning or instruction.

IN USE make a targeted clarification request

source attribution

Identification of where information came from.

IN USE preserve source attribution

working assumption

Provisional premise used while information is incomplete.

IN USE challenge a working assumption

scope boundary

Limit of the work or authority being discussed.

IN USE state the scope boundary

action owner

Named person responsible for a specified next step.

IN USE confirm the action owner

check-back

Restatement used to verify that a message was understood.

IN USE complete a check-back

Precision check

Define the scale, time window, units, and recording method. Do not use a workload label or ratio from this language exercise as an injury prediction or training prescription.

Phrases you can actually use

C Notice the language / 1 of 2

ALIGN THE MEASURE

"Are we comparing time, distance, power, or reported effort?"

PRESERVE THE DIFFERENCE

"The durations match, but the session demands differ."

CLARIFY THE SCALE

"Which effort scale was used for this entry?"

DEFINE THE WINDOW

"What dates are included in the weekly total?"

SEPARATE DATA TYPES

"That is a device estimate; this is the athlete's report."

AVOID PREDICTION

"This record alone cannot predict an injury or guarantee readiness."

ASK ABOUT CONTEXT

"What changed in the session besides the number of minutes?"

SUMMARIZE PRECISELY

"Equal time does not mean an equivalent training session."

Same

Same needs a named dimension. Same duration is narrower than same load or same effect.

Reported versus recorded by a device

These identify different sources and should not be silently merged into one supposedly objective value.

Over the week

The time window must be explicit when totals or averages are compared.

Phrases you can actually use

C Notice the language / 2 of 2

CLARIFY THE QUESTION

"Are you asking about the finding or the next step?"

INVITE CORRECTION

"Please correct me if I have misunderstood that detail."

ATTRIBUTE INFORMATION

"That is what the current record says; it has not been independently confirmed."

SEPARATE POSSIBILITY

"That is a possibility, not an agreed outcome."

CHECK SCOPE

"That decision sits outside my authority."

PIN DOWN RESPONSIBILITY

"Who is responsible for that specific next step?"

REPAIR MISUNDERSTANDING

"Let me restate the distinction more clearly."

CLOSE AN EXCHANGE

"We agree on the action, the owner, and the point for review."

Reported versus verified

Reported attributes information to a source; verified indicates it has been checked. Do not use the words interchangeably.

Could versus will

Could introduces a possibility. Will commits to an action or predicts an outcome; check whether that certainty is justified.

By versus at

By gives a latest time; at identifies a scheduled moment. State the time zone when people work across locations.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which comparison is justified by the logs?

A Equal duration but different recorded intensity
B Identical overall load
C Identical clinical effect
D Guaranteed equal recovery time

2. What does perceived exertion identify?

A A direct image of tissue damage
B A guaranteed readiness score
C The device's battery level
D The person's reported effort

3. Why clarify the weekly date range?

A Time windows never affect comparisons
B Totals need matching time windows
C Dates establish a diagnosis
D Every week has identical activity

4. Which statement overreaches?

A This wearable number guarantees no injury
B The device reports an estimate
C The sessions differ in intensity
D The durations both equal thirty minutes

Answers and explanations: page 93.

The same minutes, a different session

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

wearable metric / session RPE / weekly mileage / recovery status / intensity / monitoring limitation / work-to-rest ratio / external load / training load / Internal load

01 **Dani:** Both sessions were thirty minutes, so I entered them as the same training load in my summary.

02 **Priya:** They have the same duration, but [1] _____ is not described completely by the number of minutes alone.

03 **Dani:** One session was easy cycling, and the other was repeated hard intervals with recovery periods between them.

04 **Priya:** That difference in [2] _____ should remain visible instead of disappearing behind a shared duration figure.

05 **Dani:** I also used a perceived-effort scale, though I am not sure whether the other athlete used the same one.

06 **Priya:** Clarify the scale before comparing [3] _____. The number needs its definition and recording method attached to it.

07 **Dani:** So my report of effort is not automatically the same kind of information as the bike's power reading.

08 **Priya:** Correct. The work performed can describe [4] _____, while your response to that work is a different part of the account.

09 **Dani:** The same bike session might feel harder on another day, but that feeling alone would not tell us why.

10 **Priya:** Exactly. [5] _____ describes the response, not a complete explanation of every factor behind it.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 46.

11 **Dani:** I wrote intervals without saying how long the recovery periods were. That seems to leave an important detail out.

12 **Priya:** The [6] _____ helps describe the session, provided we use the actual recorded timings rather than inventing them.

13 **Dani:** My weekly total also starts on Sunday, while the team dashboard uses Monday through Sunday.

14 **Priya:** Then even [7] _____ needs a shared date window before you compare those totals meaningfully.

15 **Dani:** The watch gives a recovery number, but I should not describe that as a clinician saying I am ready.

16 **Priya:** A [8] _____ is not clinical clearance. Its method and limitations belong in the interpretation.

17 **Dani:** I can report the sessions and how I felt without claiming that a ratio predicts an injury.

18 **Priya:** Yes. Keep any [9] _____ explicit rather than using a precise-looking number to support an unsupported prediction.

19 **Dani:** My corrected summary is equal duration, different intensity, different activity structure, and clearly identified data sources.

20 **Priya:** That is useful. We will assess [10] _____ and any training changes through the clinical process, not by treating those two durations as equivalent sessions.

ANSWERS / EXPLANATIONS: PAGE 94

1. training load | 2. intensity | 3. session RPE | 4. external load | 5. Internal load | 6. work-to-rest ratio | 7. weekly mileage | 8. wearable metric | 9. monitoring limitation | 10. recovery status

The weekly totals cover different days

F Say it

Two logs both show 20 kilometers. Log A covers Monday to Friday; Log B covers Monday to Sunday. No intensity information is supplied.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

different / equal / absent / demand

Athlete: "The distance totals are [1] _____."

Clinician: "The recording windows are [2] _____."

Athlete: "Intensity information is [3] _____."

Clinician: "Equal distance does not establish equivalent [4] _____."

ANSWERS / EXPLANATIONS: PAGE 94

1. equal | 2. different | 3. absent | 4. demand

Rehearse with a fixed script

1. Read the corrected comparison and emphasize the dimension attached to same.

2. Swap roles and distinguish recorded work, reported effort, and device estimates without prescribing training.

3. Complete the four-line follow-up with the supplied choices. Check the explanations, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

An image is not a return-to-play decision

A Read the situation

Explain the distinction between a structural report, symptoms, function, and a participation decision.

Athlete Niko reads a fictional imaging report describing a finding that the reporting clinician says needs clinical correlation. No specific structure, diagnosis, severity, or treatment recommendation is supplied. Dr Bell explains what correlation means and why neither an alarming word nor a reassuring image phrase alone establishes symptoms, function, or sport clearance.

PEOPLE / Niko: Athlete | Dr Bell: Sports physician

TRANSLATE THE QUESTION THE TEST ANSWERED

Patients may read a report before discussing it with a clinician. Explain what an imaging finding can describe without making it the whole story or dismissing the concern it raises.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What does the report request?

A Immediate unrestricted sport
B A specific operation
C A guaranteed prognosis
D Clinical correlation

2. Which detail is deliberately not supplied?

A A named structural diagnosis
B A report phrase
C The athlete's question
D The physician's explanatory role

3. What cannot be concluded from this exercise?

A The athlete's sport clearance
B That an imaging report exists
C That context matters
D That Niko wants clarification

Answers and explanations: page 95.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

imaging report

Written interpretation of an imaging examination.

IN USE review the imaging report

radiograph

Image commonly produced using X-rays.

IN USE explain a radiograph

magnetic resonance imaging

Imaging method using magnetic fields and radio waves.

IN USE discuss magnetic resonance imaging

diagnostic ultrasound

Imaging using sound waves for the relevant clinical examination.

IN USE explain diagnostic ultrasound

structural finding

Feature of anatomy described by examination or imaging.

IN USE interpret a structural finding

incidental finding

Finding discovered outside the primary question prompting the examination.

IN USE explain an incidental finding

clinical correlation

Interpretation of a finding alongside history, examination, and other relevant information.

IN USE seek clinical correlation

clinical significance

Meaning or importance of a finding for the actual clinical situation.

IN USE clarify clinical significance

imaging limitation

Constraint on what an examination can show or establish.

IN USE state an imaging limitation

report impression

Summarized interpretation section of an imaging report.

IN USE explain the report impression

comparison study

Earlier examination used as a reference for the current interpretation.

IN USE identify the comparison study

image quality

Technical adequacy of images for the question being assessed.

IN USE describe image quality

Precision check

Clinical correlation means the finding must be interpreted with the relevant clinical information. It does not mean the report proves the cause of symptoms, mandates treatment, or authorizes sport.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

artifact

Image feature caused by technical or acquisition factors rather than the assumed anatomy.

IN USE identify a possible artifact

differential diagnosis

Set of possible explanations evaluated through clinical reasoning.

IN USE explain a differential diagnosis

symptom generator

Structure or process considered a possible source of symptoms.

IN USE assess a possible symptom generator

functional limitation

Restriction in performing a relevant activity.

IN USE describe a functional limitation

prognosis

Expected course or outcome with appropriate uncertainty.

IN USE discuss prognosis cautiously

shared interpretation

Explanation bringing the relevant findings and perspectives together.

IN USE support shared interpretation

clarification request

Question that resolves an unclear meaning or instruction.

IN USE make a targeted clarification request

source attribution

Identification of where information came from.

IN USE preserve source attribution

working assumption

Provisional premise used while information is incomplete.

IN USE challenge a working assumption

scope boundary

Limit of the work or authority being discussed.

IN USE state the scope boundary

action owner

Named person responsible for a specified next step.

IN USE confirm the action owner

check-back

Restatement used to verify that a message was understood.

IN USE complete a check-back

Precision check

Do not invent a scan result or label a finding harmless from this exercise. An incidental finding is not automatically irrelevant; its actual significance needs appropriate review.

Phrases you can actually use

C Notice the language / 1 of 2

EXPLAIN CORRELATION

"We need to connect this finding with the history and examination."

SEPARATE STRUCTURE AND FUNCTION

"The image and your activity limitations answer different questions."

AVOID A LEAP

"That phrase alone does not identify the cause of your symptoms."

KEEP LIMITS VISIBLE

"The examination has limits that the report should describe."

EXPLAIN INCIDENTAL

"It was found outside the main question; that does not automatically mean unimportant."

ASK WHAT MATTERS

"Which part of the report is most unclear to you?"

PRESERVE UNCERTAINTY

"We should not turn a possible explanation into a confirmed one."

SEPARATE PERMISSION

"An imaging phrase is not itself sport clearance."

Suggests versus confirms

Suggests identifies a supported possibility; confirms asserts a stronger conclusion and needs the relevant evidence.

Incidental

Incidental describes how the finding arose in relation to the original question, not a universal judgment of harmlessness.

Correlate with

Correlate with requests interpretation alongside other information, not simple repetition of the report wording.

Phrases you can actually use

C Notice the language / 2 of 2

CLARIFY THE QUESTION

"Are you asking about the finding or the next step?"

INVITE CORRECTION

"Please correct me if I have misunderstood that detail."

ATTRIBUTE INFORMATION

"That is what the current record says; it has not been independently confirmed."

SEPARATE POSSIBILITY

"That is a possibility, not an agreed outcome."

CHECK SCOPE

"That decision sits outside my authority."

PIN DOWN RESPONSIBILITY

"Who is responsible for that specific next step?"

REPAIR MISUNDERSTANDING

"Let me restate the distinction more clearly."

CLOSE AN EXCHANGE

"We agree on the action, the owner, and the point for review."

Reported versus verified

Reported attributes information to a source; verified indicates it has been checked. Do not use the words interchangeably.

Could versus will

Could introduces a possibility. Will commits to an action or predicts an outcome; check whether that certainty is justified.

By versus at

By gives a latest time; at identifies a scheduled moment. State the time zone when people work across locations.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which explanation best fits clinical correlation?

A Ignore the athlete's symptoms
B Choose treatment from one isolated word
C Interpret the finding with the history and examination
D Treat the report as complete sport clearance

2. Which conclusion does incidental not automatically support?

A The finding was outside the primary question
B The context matters
C The report needs explanation
D The finding is harmless and needs no review

3. Which pair should remain distinct?

A Report and written report
B Scan and imaging examination
C Question and inquiry
D Structural finding and functional limitation

4. Which response handles uncertainty accurately?

A No image has any clinical value
B This possible explanation needs clinical review
C The report guarantees the cause
D Every image phrase is a treatment instruction

Answers and explanations: page 95.

An image is not a return-to-play decision

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

Clinical correlation / clinical significance / shared interpretation / functional limitation / imaging limitation / artifact / prognosis / incidental finding / structural finding / report impression

01 **Niko:** I read the report before the appointment, and the phrase clinical correlation made me think something had been missed.

02 **Dr Bell:** [1] _____ means we interpret the finding alongside the history, examination, and other relevant information, not that the phrase itself proves an error.

03 **Niko:** I saw one unfamiliar word and began treating it as the explanation for every symptom I have.

04 **Dr Bell:** A [2] _____ is not automatically the cause of every symptom. We need the actual clinical context before making that connection.

05 **Niko:** Does the summary at the end carry a different role from the detailed description above it?

06 **Dr Bell:** The [3] _____ summarizes the interpretation, but it still needs to be read with the details and any stated limitations.

07 **Niko:** If the report mentions an incidental finding, can I assume it has no importance at all?

08 **Dr Bell:** No. An [4] _____ is outside the primary question that prompted the examination; its actual significance still needs appropriate review.

09 **Niko:** I also want to understand why my ability to train is discussed separately from what an image shows.

10 **Dr Bell:** A [5] _____ describes difficulty performing an activity. It is a different kind of information from the structure described in a scan.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 55.

11 **Niko:** So a reassuring-sounding phrase would not by itself give me permission to return to every sporting demand.

12 **Dr Bell:** Correct. A [6] _____ brings the relevant information together; neither a reassuring phrase nor an alarming word supplies the entire participation decision.

13 **Niko:** The report also mentions limits. I thought any modern scan would answer every possible question about the injury.

14 **Dr Bell:** An [7] _____ tells us what the examination cannot reliably show or establish. Different methods answer different questions.

15 **Niko:** Could a technical feature in an image be mistaken for something anatomical if the context is lost?

16 **Dr Bell:** That is one reason an [8] _____ must be interpreted by the appropriate imaging professional rather than diagnosed from a cropped picture.

17 **Niko:** I would like an explanation of the actual report, not a general promise that every uncertain phrase is harmless.

18 **Dr Bell:** We can do that while keeping [9] _____ separate from how dramatic a term sounds when read without an explanation.

19 **Niko:** And any forecast about recovery should state what is known and what remains uncertain in my assessment.

20 **Dr Bell:** Exactly. A [10] _____ is not a guarantee extracted from one report phrase. It belongs to the actual clinical discussion.

ANSWERS / EXPLANATIONS: PAGE 96

1. Clinical correlation | 2. structural finding | 3. report impression | 4. incidental finding | 5. functional limitation | 6. shared interpretation | 7. imaging limitation | 8. artifact | 9. clinical significance | 10. prognosis

Suggests became confirms

F Say it

A referral summary changed the report phrase suggests a possible explanation to confirms the cause. The original report required clinical correlation; no further assessment is supplied.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

certainty / suggests / confirms / remains

Clinician: "The original report said [1] _____."

Colleague: "The summary incorrectly changed that to [2] _____."

Clinician: "The need for clinical correlation [3] _____."

Colleague: "I will correct the overstated [4] _____."

ANSWERS / EXPLANATIONS: PAGE 96

1. suggests | 2. confirms | 3. remains | 4. certainty

Rehearse with a fixed script

1. Read the corrected explanation and distinguish finding, significance, function, and clearance.

2. Change roles and explain clinical correlation without inventing a diagnosis or treatment recommendation.

3. Complete the four-line follow-up with the supplied choices. Check the explanations, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

The final is a date, not a criterion

A Read the situation

Communicate a conditional participation decision while acknowledging competitive pressure and uncertainty.

Athlete Imani hopes to play in a final in two weeks. The actual rehabilitation team has not yet authorized competition; a review of the relevant functional and sport-specific criteria is pending. Sports physician Dr Vale explains that the event date is not a clinical criterion. No test score, recovery timetable, or specific clearance standard is supplied. Imani asks how to explain the position to the coach without sounding uncommitted.

PEOPLE / Imani: Athlete | Dr Vale: Sports physician

ACKNOWLEDGE THE STAKES WITHOUT BARGAINING AWAY THE DECISION

The athlete may fear losing a role or disappointing teammates. Recognize that pressure while keeping authorization tied to the actual assessment and responsible clinician.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What is the current competition status?

A Not yet authorized
B Unrestricted clearance
C Guaranteed selection
D Approved because the final is near

2. What is pending?

A A known winning result
B A supplied normal test score
C A promised two-week recovery
D Review of relevant criteria

3. What does two weeks identify?

A A universal clearance standard
B A prescribed rehabilitation dose
C Time until the final
D Guaranteed recovery duration

Answers and explanations: page 97.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

return to participation

Resumption of some sporting involvement under the relevant limits.

IN USE distinguish return to participation

return to performance

Regaining the desired level of sporting performance.

IN USE distinguish return to performance

competition readiness

Suitability for the relevant competitive demands under actual assessment.

IN USE discuss competition readiness

sport-specific demand

Requirement particular to the athlete's sport or role.

IN USE identify sport-specific demands

functional testing

Assessment of relevant task performance under the actual clinical process.

IN USE explain functional testing

criteria-based decision

Decision linked to specified evidence or conditions rather than time alone.

IN USE communicate a criteria-based decision

time-based milestone

Point defined by elapsed time.

IN USE distinguish a time-based milestone

psychological readiness

Mental preparedness relevant to returning to activity.

IN USE discuss psychological readiness

fear of reinjury

Concern about sustaining injury again.

IN USE acknowledge fear of reinjury

confidence in movement

Person's trust in performing a movement or task.

IN USE explore confidence in movement

graded exposure

Planned increase in exposure to relevant demands within an appropriate plan.

IN USE clarify graded exposure

movement quality

Characteristics of how a task is performed.

IN USE assess movement quality

Precision check

An event date and time since injury are not interchangeable with satisfied criteria. Participation, competition, and performance can describe different demands and permissions.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

capacity

Ability to meet a specified demand under the relevant assessment.

IN USE describe assessed capacity

performance demand

Requirement needed for the intended level of sporting performance.

IN USE specify the performance demand

selection pressure

Pressure associated with team or event selection.

IN USE acknowledge selection pressure

conditional authorization

Permission dependent on stated conditions.

IN USE communicate conditional authorization

decision rationale

Explanation of the basis for a decision.

IN USE explain the decision rationale

review outcome

Result of the actual review.

IN USE communicate the review outcome

clarification request

Question that resolves an unclear meaning or instruction.

IN USE make a targeted clarification request

source attribution

Identification of where information came from.

IN USE preserve source attribution

working assumption

Provisional premise used while information is incomplete.

IN USE challenge a working assumption

scope boundary

Limit of the work or authority being discussed.

IN USE state the scope boundary

action owner

Named person responsible for a specified next step.

IN USE confirm the action owner

check-back

Restatement used to verify that a message was understood.

IN USE complete a check-back

Precision check

This case supplies no pass score or clearance checklist. It practices conditional language, not a universal return-to-sport standard or an individualized prediction.

Phrases you can actually use

C Notice the language / 1 of 2

ACKNOWLEDGE THE GOAL

"I understand why this final matters to you."

STATE THE PRESENT POSITION

"Competition has not yet been authorized."

SEPARATE THE CALENDAR

"The date tells us when the event happens, not whether the criteria are met."

EXPLAIN CONDITIONS

"The decision depends on the actual review and relevant findings."

AVOID A PROMISE

"I cannot turn the two-week interval into a guaranteed recovery time."

KEEP ROLES CLEAR

"Team selection and clinical authorization are different decisions."

SUPPORT THE MESSAGE

"You can state the current limit without apologizing for following the plan."

CLOSE WITH THE NEXT STEP

"We will communicate the actual review outcome and its scope."

If versus when

If preserves a condition that may not be met; when can imply that authorization is certain to occur.

Ready for what

Ready needs a named demand: limited participation, competition, and prior performance are different targets.

Not yet

Not yet identifies the current status without committing to a future date or result.

Phrases you can actually use

C Notice the language / 2 of 2

CLARIFY THE QUESTION

"Are you asking about the finding or the next step?"

INVITE CORRECTION

"Please correct me if I have misunderstood that detail."

ATTRIBUTE INFORMATION

"That is what the current record says; it has not been independently confirmed."

SEPARATE POSSIBILITY

"That is a possibility, not an agreed outcome."

CHECK SCOPE

"That decision sits outside my authority."

PIN DOWN RESPONSIBILITY

"Who is responsible for that specific next step?"

REPAIR MISUNDERSTANDING

"Let me restate the distinction more clearly."

CLOSE AN EXCHANGE

"We agree on the action, the owner, and the point for review."

Reported versus verified

Reported attributes information to a source; verified indicates it has been checked. Do not use the words interchangeably.

Could versus will

Could introduces a possibility. Will commits to an action or predicts an outcome; check whether that certainty is justified.

By versus at

By gives a latest time; at identifies a scheduled moment. State the time zone when people work across locations.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which sentence preserves the condition?

A When you are cleared next week, play the final
B The final itself proves readiness
C Any participation means full performance
D If the actual review supports authorization, the team will communicate its scope

2. Which distinction helps answer the coach?

A Final and any calendar date
B Athlete and team member
C Clinical authorization and team selection
D Medical review and a jersey color

3. Which reply acknowledges pressure without promising recovery?

A Confidence alone settles readiness
B I understand the final matters; the review still determines the decision
C You will definitely play because you care
D Missing a final means poor commitment

4. What is not supplied in this exercise?

A An event date
B A universal pass score
C A pending review
D A competition goal

Answers and explanations: page 97.

The final is a date, not a criterion

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

review outcome / decision rationale / sport-specific demand / time-based milestone / Return to participation / criteria-based decision / conditional authorization / Functional testing / Psychological readiness / selection pressure

01 **Imani:** The final is in two weeks, and I am worried that saying I am not cleared will sound like a lack of commitment.

02 **Dr Vale:** I understand the [1] _____. Caring about the team does not require you to describe an authorization that has not been given.

03 **Imani:** Can the two weeks become our target, or would that make the date sound more certain than the assessment allows?

04 **Dr Vale:** It is a [2] _____, not proof that the relevant criteria will be met. I cannot promise recovery by that date.

05 **Imani:** Then what is the distinction between returning to some activity and returning to the level I played at before?

06 **Dr Vale:** [3] _____ may describe limited involvement, while competition and your previous performance can involve different demands.

07 **Imani:** I do not want a note about limited activity to be repeated as permission to play an entire match.

08 **Dr Vale:** That is why [4] _____ needs its conditions and scope attached whenever it is communicated to the team.

09 **Imani:** The coach asked whether I passed the functional tests, but I do not have a completed review outcome to report.

10 **Dr Vale:** Then say that. [5] _____ is part of the actual process; we should not insert a pass score that has not been supplied.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 64.

11 **Imani:** I can also feel confident in everyday movement and still feel uncertain about a full-speed change of direction.

12 **Dr Vale:** A [6] _____ is not necessarily equivalent to an everyday activity. The real assessment addresses the relevant demands.

13 **Imani:** Some of my hesitation is fear of another injury, but I do not want it dismissed as being weak.

14 **Dr Vale:** [7] _____ deserves a respectful discussion alongside the other relevant information, not a judgment about your character.

15 **Imani:** How can I explain the position in one clear sentence without sounding as though I am negotiating the decision?

16 **Dr Vale:** You can say the [8] _____ is pending and competition has not yet been authorized. That is factual, not an apology.

17 **Imani:** And if the coach chooses another player, that selection decision is separate from the clinical question.

18 **Dr Vale:** Correct. We should explain the [9] _____ clearly while leaving team selection to the appropriate team process.

19 **Imani:** Then the message is that I want to return, but desire and the calendar do not establish permission.

20 **Dr Vale:** Exactly. A [10] _____ preserves the actual evidence and responsibility, without promising when the required decision will be made.

ANSWERS / EXPLANATIONS: PAGE 98

1. selection pressure | 2. time-based milestone | 3. Return to participation | 4. conditional authorization | 5. Functional testing | 6. sport-specific demand | 7. Psychological readiness | 8. review outcome | 9. decision rationale | 10. criteria-based decision

A conditional note was shortened

F Say it

A clinical note allows only the specified activity under supervision. A team summary changes it to unrestricted training. No wider authorization exists.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

scope / unrestricted / condition / supervision

Clinician: "The note requires [1] _____."

Coach: "The summary incorrectly said [2] _____ training."

Clinician: "That change removed a [3] _____."

Coach: "I will restore the actual [4] _____."

ANSWERS / EXPLANATIONS: PAGE 98

1. supervision | 2. unrestricted | 3. condition | 4. scope

Rehearse with a fixed script

1. Read the corrected exchange and contrast if with when in the authorization statements.

2. Swap roles and give the one-sentence status update without adding a return date or test result.

3. Complete the four-line follow-up with the supplied choices. Check the explanations, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

The coach needs the limit, not the whole chart

A Read the situation

Clarify the recipient, purpose, and authorized scope of a sports-medicine update.

Team coordinator Riley requests athlete Casey's complete medical chart to plan a practice session. Clinician Dr Shah explains that the team needs the actual authorized participation information, while access to health details follows the relevant consent, law, and institutional policy. Casey's permission for a limited participation update is documented; permission for unrestricted chart sharing is not. No diagnosis or specific restriction is supplied in the exercise.

PEOPLE / Riley: Team coordinator | Dr Shah: Clinician

ROLE CLARITY REDUCES FRICTION

A colleague can have a legitimate planning need without being entitled to every clinical detail. Explain the useful information that can be shared through the actual authorized process.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What does the coordinator need for the stated task?

A A guessed diagnosis
B Unrestricted personal records
C Authorized participation information
D Every historical clinical note

2. What permission is documented?

A A limited participation update
B Unlimited chart distribution
C Public posting of the diagnosis
D Permission to add new restrictions

3. What must govern access to health details?

A The match result
B A general assumption about teammates
C Relevant consent, law, and institutional policy
D The coordinator's curiosity

Answers and explanations: page 99.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

confidentiality

Obligation to protect information under the applicable professional and legal rules.

IN USE maintain confidentiality

authorized disclosure

Sharing permitted under the relevant consent and other applicable requirements.

IN USE confirm authorized disclosure

participation restriction

Limit on an activity under the actual authorized plan.

IN USE communicate a participation restriction

need-to-know access

Access limited to information relevant to an authorized role and purpose.

IN USE apply need-to-know access

clinical record

Recorded information about assessment, care, and relevant health matters.

IN USE protect the clinical record

release authorization

Documented permission for specified information sharing under applicable rules.

IN USE verify a release authorization

recipient

Person or organization receiving the information.

IN USE identify the intended recipient

disclosure purpose

Reason for a particular information exchange.

IN USE specify the disclosure purpose

data minimization

Limiting shared information to what is appropriate for the purpose and requirements.

IN USE apply data minimization

secure channel

Approved communication route with appropriate protections.

IN USE use a secure channel

role-based access

Access permissions linked to an authorized work role.

IN USE check role-based access

athlete representation

Support or advocacy arrangement acting for an athlete within defined authority.

IN USE clarify athlete representation

Precision check

A legitimate planning purpose does not automatically authorize access to the whole chart. Share the actual permitted information through the approved route and keep the athlete's documented permission and applicable rules in view.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

team liaison

Contact coordinating communication between the team and relevant services.

IN USE identify the team liaison

conflict of interest

Situation in which competing interests may affect professional judgment.

IN USE disclose a conflict of interest

professional independence

Ability to exercise appropriate professional judgment without improper pressure.

IN USE preserve professional independence

status update

Communication of the current relevant position.

IN USE provide an accurate status update

audit trail

Traceable record of actions, access, or changes.

IN USE retain an audit trail

information governance

Rules and processes controlling information handling.

IN USE follow information governance

clarification request

Question that resolves an unclear meaning or instruction.

IN USE make a targeted clarification request

source attribution

Identification of where information came from.

IN USE preserve source attribution

working assumption

Provisional premise used while information is incomplete.

IN USE challenge a working assumption

scope boundary

Limit of the work or authority being discussed.

IN USE state the scope boundary

action owner

Named person responsible for a specified next step.

IN USE confirm the action owner

check-back

Restatement used to verify that a message was understood.

IN USE complete a check-back

Precision check

Privacy law, consent requirements, safeguarding duties, and institutional roles differ. This language exercise does not establish a universal disclosure rule or invent a clinical restriction.

Phrases you can actually use

C Notice the language / 1 of 2

CLARIFY THE TASK

"What decision do you need this information to support?"

IDENTIFY THE USEFUL UPDATE

"The participation information addresses the practice-planning question."

SET THE LIMIT

"Permission for a limited update is not unrestricted access to the chart."

CHECK THE RECIPIENT

"Who will receive the message, and will it be forwarded?"

PRESERVE AUTHORITY

"Clinical judgment is separate from pressure to field a player."

CHOOSE THE ROUTE

"We should use the approved communication channel."

AVOID DISCLOSURE BY HABIT

"Let us verify the authorization rather than copy the full record."

CLOSE CONSTRUCTIVELY

"I will clarify what can be shared for this specific purpose."

Need versus entitlement

A practical need should be clarified; it does not automatically establish permission for all requested information.

Limited to

Limited to marks a boundary that should remain attached when a message is forwarded or summarized.

Can share

Can share depends on the actual authorization and applicable requirements, not merely technical ability to send a file.

Phrases you can actually use

C Notice the language / 2 of 2

CLARIFY THE QUESTION

"Are you asking about the finding or the next step?"

INVITE CORRECTION

"Please correct me if I have misunderstood that detail."

ATTRIBUTE INFORMATION

"That is what the current record says; it has not been independently confirmed."

SEPARATE POSSIBILITY

"That is a possibility, not an agreed outcome."

CHECK SCOPE

"That decision sits outside my authority."

PIN DOWN RESPONSIBILITY

"Who is responsible for that specific next step?"

REPAIR MISUNDERSTANDING

"Let me restate the distinction more clearly."

CLOSE AN EXCHANGE

"We agree on the action, the owner, and the point for review."

Reported versus verified

Reported attributes information to a source; verified indicates it has been checked. Do not use the words interchangeably.

Could versus will

Could introduces a possibility. Will commits to an action or predicts an outcome; check whether that certainty is justified.

By versus at

By gives a latest time; at identifies a scheduled moment. State the time zone when people work across locations.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which reply is both useful and appropriately bounded?

A I will invent a restriction so no details are needed
B Planning never needs any information
C Let us confirm the authorized participation update for practice planning
D You may have every record because you are on the team

2. What does documented limited permission not establish?

A The need to check scope
B The identity of the relevant athlete
C Unrestricted chart access
D A defined update purpose

3. Which question should precede forwarding?

A Is the file small enough?
B Is the team winning?
C Would a shorter subject line look better?
D Is the new recipient and purpose authorized?

4. Which distinction protects professional judgment?

A A record and its page count
B Practice and a calendar entry
C Clinical authorization and team selection pressure
D A clinician's title and font size

Answers and explanations: page 99.

The coach needs the limit, not the whole chart

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

Confidentiality / status update / professional independence / release authorization / secure channel / disclosure purpose / recipient / audit trail / Role-based access / data minimization

01 **Riley:** Could you send Casey's complete chart? I need to organize tomorrow's practice and do not want to miss anything.

02 **Dr Shah:** What decision do you need to make? Clarifying the [1] _____ will help us identify the relevant information rather than defaulting to the whole record.

03 **Riley:** I need the actual participation limits so the practice plan does not contradict the clinical instructions.

04 **Dr Shah:** That calls for an authorized [2] _____. We should not assume that it requires every historical clinical detail.

05 **Riley:** Casey agreed to a limited participation update. Does that permission let me forward the whole chart to the assistants?

06 **Dr Shah:** No. We need to preserve the scope of the [3] _____ and the applicable rules, including the actual recipients and purpose.

07 **Riley:** Then I should identify who will receive the message instead of using an expanding group chat by habit.

08 **Dr Shah:** Correct. Each [4] _____ and the communication route need to fit the relevant authorization and institutional requirements.

09 **Riley:** I do not need the diagnostic history simply to understand the participation information that has been authorized.

10 **Dr Shah:** That is the point of [5] _____. It keeps the message useful without adding unrelated personal detail.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 73.

11 **Riley:** Would the ordinary team messaging app be acceptable, or should we check the service's actual requirements first?

12 **Dr Shah:** We should use the approved [6] _____ rather than assume that convenience establishes an appropriate route for health information.

13 **Riley:** The coach is under pressure to field the strongest team. I do not want that to affect how the clinical message is written.

14 **Dr Shah:** The clinical decision needs [7] _____. We can communicate clearly without changing the authorization to satisfy a selection preference.

15 **Riley:** And my role as coordinator does not automatically give me every access permission held by the clinical team.

16 **Dr Shah:** Exactly. [8] _____ depends on the actual authorized role, not simply belonging to the same organization.

17 **Riley:** I will tell the assistants that the participation update is being clarified, without adding a diagnosis or restriction from this conversation.

18 **Dr Shah:** Good. We should also retain the appropriate [9] _____ of what was shared, with whom, and under which approved process.

19 **Riley:** Then we can support practice planning while respecting Casey's permission and the actual information-handling rules.

20 **Dr Shah:** Yes. [10] _____ and useful coordination are compatible when the purpose, authority, and scope remain clear.

ANSWERS / EXPLANATIONS: PAGE 100

1. disclosure purpose | 2. status update | 3. release authorization | 4. recipient | 5. data minimization | 6. secure channel | 7. professional independence | 8. Role-based access | 9. audit trail | 10. Confidentiality

A new recipient was added

F Say it

A participation update was authorized for the team liaison. Before forwarding it to an external sponsor, the coordinator checks the actual authorization; no sponsor disclosure has been approved.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

liaison / scope / recipient / approved

Coordinator: "The authorized recipient is the team [1] _____."

Clinician: "The sponsor is a different [2] _____."

Coordinator: "Sponsor disclosure has not been [3] _____."

Clinician: "We must check the actual authorization [4] _____."

ANSWERS / EXPLANATIONS: PAGE 100

1. liaison | 2. recipient | 3. approved | 4. scope

Rehearse with a fixed script

1. Read the corrected exchange and identify purpose, recipient, scope, and channel.

2. Swap roles and provide a bounded response to the chart request without inventing a diagnosis or restriction.

3. Complete the four-line follow-up with the supplied choices. Check the explanations, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Clarifying a therapeutic use exemption request

E More workplace conversations / 1-10

Distinguish medical treatment questions from anti-doping authorization and application status.

Athlete Remy has a prescription from the treating clinician and asks team physician Dr Noor whether that automatically establishes permission under the relevant anti-doping rules. The actual medicine, sport, athlete classification, and rules are not supplied. A therapeutic use exemption question must be checked through the responsible anti-doping process; no medicine change or exemption outcome is authorized by this case.

PEOPLE / Remy: Athlete | Dr Noor: Team physician

Use each bank entry once, without changing its form. Answers follow turn 20.

WORD + PHRASE BANK

responsible anti-doping organization / Application receipt / pending decision / therapeutic use exemption / authorization distinction / athlete classification

01 **Remy:** I have a prescription from my treating clinician. Does that automatically settle the anti-doping permission question as well?

02 **Dr Noor:** Not automatically. The clinical prescription and the applicable sporting requirements are distinct questions that need their proper review.

03 **Remy:** I have heard of a therapeutic use exemption, but I do not know whether my actual situation requires one.

04 **Dr Noor:** A [1] _____ question needs the actual medicine, rules, athlete classification, and responsible organization's process.

05 **Remy:** Then I should not rely on a teammate who said every prescribed medicine is automatically permitted in every sport.

06 **Dr Noor:** Correct. We need the current applicable requirements rather than a general claim that ignores the actual details.

07 **Remy:** The online form says my request was received. I was about to describe that as exemption approved.

08 **Dr Noor:** [2] _____ is not approval. We should check the actual status and any outstanding information through the responsible process.

09 **Remy:** There is also a question about the level at which I compete. Why does that matter to the inquiry?

10 **Dr Noor:** The [3] _____ helps identify the relevant requirements and organization. We should use the actual category, not assume it from the team name.

The conversation, continued

E More workplace conversations / 11-20

Continue with the word bank on page 76.

11 **Remy:** I can provide the current medicine information and the documents requested through the proper channel.

12 **Dr Noor:** Good. Keep the clinical information accurate and limited to the authorized purpose; missing details should not be invented to complete a form.

13 **Remy:** If the status is pending, I should say pending rather than reassure the coach that every requirement is settled.

14 **Dr Noor:** Exactly. A [4] _____ needs to remain visible, along with who is responsible for answering the regulatory question.

15 **Remy:** Should I change my medicine because the administrative question is unresolved?

16 **Dr Noor:** Do not use this conversation as a medicine-change instruction. Treatment questions belong with the appropriate treating professional and actual clinical process.

17 **Remy:** So the next step is an accurate inquiry, not a guess about a banned list or an exemption outcome.

18 **Dr Noor:** Yes. The [5] _____ needs to be identified so the question goes to the correct authority.

19 **Remy:** I will explain the status as a request requiring review and avoid saying the prescription itself grants sporting permission.

20 **Dr Noor:** That preserves the [6] _____ without interrupting necessary clinical care or inventing a regulatory result.

ANSWERS / EXPLANATIONS: PAGE 101

1. therapeutic use exemption | 2. Application receipt | 3. athlete classification | 4. pending decision | 5. responsible anti-doping organization | 6. authorization distinction

Received is not approved

F Say it in a new situation

An exemption portal confirms receipt of an application. It asks for further information and gives no approval decision. The athlete reports the status to the team.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

status / requested / given / received

Athlete: "The application has been [1] _____."

Clinician: "Further information is still [2] _____."

Athlete: "Approval has not been [3] _____."

Clinician: "We should report the actual application [4] _____."

ANSWERS / EXPLANATIONS: PAGE 101

1. received | 2. requested | 3. given | 4. status

Read, switch, repeat

1. Check the four answers and read the complete exchange aloud.

2. Switch roles. Read it again with the word bank covered.

3. Return to the extended conversation. Read turns 1-10, then switch roles for turns 11-20. Use the corrected expressions.

SOLO PRACTICE

Read both roles. Pause briefly at each change of speaker. Repeat any line that needed an answer check.

Handing over care before overseas travel

E More workplace conversations / 1-10

Transfer verified rehabilitation information across teams and time zones without losing responsibility or emergency routes.

Physiotherapist Erin hands athlete Jules's care to destination clinician Dr Kim before an overseas event. The current rehabilitation plan is available through an authorized channel. Travel dates are confirmed, but the receiving appointment is only requested. No new participation authorization or exercise instruction is supplied. They clarify document versions, contact times, and the pending appointment.

PEOPLE / Erin: Referring physiotherapist | Dr Kim: Destination clinician

Use each bank entry once, without changing its form. Answers follow turn 20.

WORD + PHRASE BANK

booking status / handoff responsibility / document provenance / continuity of care / travel purpose / time zone

01 **Erin:** Jules travels on the confirmed itinerary, but the receiving appointment is still a request rather than a completed booking.

02 **Dr Kim:** Thank you for separating those. Travel confirmation does not mean our clinical service has already accepted an appointment time.

03 **Erin:** I will send the current rehabilitation plan through the authorized channel with its date and revision clearly identified.

04 **Dr Kim:** That [1] _____ matters because the athlete may also carry an older handout with different wording.

05 **Erin:** The plan does not authorize a new activity simply because the athlete is traveling to a sporting event.

06 **Dr Kim:** Agreed. The [2] _____ is not a substitute for the actual participation information and clinical decision.

07 **Erin:** I can identify the clinician responsible for the present plan and the outstanding review question.

08 **Dr Kim:** Please do, and make the [3] _____ explicit while the receiving arrangements are still being confirmed.

09 **Erin:** We also need a time for questions before departure. The proposed message simply says call at nine.

10 **Dr Kim:** Add the date and [4] _____. Otherwise the same clock time could refer to different moments in our locations.

The conversation, continued

E More workplace conversations / 11-20

Continue with the word bank on page 79.

11 **Erin:** I will also distinguish a routine professional contact from the athlete's actual urgent-care route at the destination.

12 **Dr Kim:** Yes. A routine inbox should not become the assumed emergency route merely because it appears on the handoff sheet.

13 **Erin:** The athlete needs a clear account of the appointment status, rather than discovering on arrival that nobody confirmed it.

14 **Dr Kim:** I will check our [5] _____ and tell you which arrangements we can actually confirm.

15 **Erin:** If the appointment time changes, we should update the athlete and both relevant teams rather than only our own calendar.

16 **Dr Kim:** Exactly. A corrected internal schedule does not automatically reach the person relying on the appointment.

17 **Erin:** I will keep the shared information limited to the purpose and actual permission for this clinical handoff.

18 **Dr Kim:** That supports [6] _____ while respecting the applicable information-sharing requirements in both settings.

19 **Erin:** Then the open items are appointment confirmation, the named contact time, and receipt of the verified current plan.

20 **Dr Kim:** Correct. We will close those items explicitly without adding a diagnosis, exercise change, or sport authorization that has not been made.

ANSWERS / EXPLANATIONS: PAGE 102

1. document provenance | 2. travel purpose | 3. handoff responsibility | 4. time zone | 5. booking status | 6. continuity of care

Nine in which time zone?

F Say it in a new situation

A handoff proposes a call at 09:00 but omits the date and time zone. Both teams agree the call is not yet fully specified. The clinical plan is unchanged.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

specified / zone / details / unchanged

Referrer: "The call still needs a date and time [1] _____."

Receiver: "The proposed contact time is not fully [2] _____."

Referrer: "The clinical plan remains [3] _____."

Receiver: "We will confirm the complete call [4] _____."

ANSWERS / EXPLANATIONS: PAGE 102

1. zone | 2. specified | 3. unchanged | 4. details

Read, switch, repeat

1. Check the four answers and read the complete exchange aloud.

2. Switch roles. Read it again with the word bank covered.

3. Return to the extended conversation. Read turns 1-10, then switch roles for turns 11-20. Use the corrected expressions.

SOLO PRACTICE

Read both roles. Pause briefly at each change of speaker. Repeat any line that needed an answer check.

Discussing a return setback without assigning blame

E More workplace conversations / 1-10

Describe a reported difficulty during rehabilitation and request review without diagnosing failure or concealing the concern.

Athlete Noor reports difficulty during a permitted rehabilitation task and fears being labeled unmotivated. Physiotherapist Alex asks what happened and keeps the actual clinical response separate from performance pressure. The task, symptoms, measurements, and clinical interpretation are not supplied. They agree to report the relevant facts to the responsible clinician through the current plan's route.

PEOPLE / Noor: Athlete | Alex: Physiotherapist

Use each bank entry once, without changing its form. Answers follow turn 20.

WORD + PHRASE BANK

responsible reviewer / event description / Performance pressure / task difficulty / plan clarification / nonjudgmental reporting

01 **Noor:** I had difficulty during the task, but I almost left it out because I thought everyone would call it a setback I caused.

02 **Alex:** I would rather hear an accurate account than have you hide a concern to make the progress report look better.

03 **Noor:** I can describe what happened, but I do not know whether it changes the injury or the plan.

04 **Alex:** Keep the [1] _____ separate from its clinical meaning. We need the actual facts before the appropriate review can interpret them.

05 **Noor:** The coach keeps asking whether I am trying hard enough, and I worry that uncertainty sounds like an excuse.

06 **Alex:** [2] _____ can make this harder to discuss, but it should not determine what you feel able to report.

07 **Noor:** Should I say the rehabilitation failed, or does that label go beyond what I actually know?

08 **Alex:** It goes beyond the facts. A reported [3] _____ does not by itself establish failure of the plan or a new injury.

09 **Noor:** I also do not want the summary to say everything was fine just because the details are not yet interpreted.

10 **Alex:** Correct. Uncertainty is not reassurance. We should describe what is known and use the actual clinical response route.

The conversation, continued

E More workplace conversations / 11-20

Continue with the word bank on page 82.

11 **Noor:** I can identify when the difficulty happened and what I was doing, without making up a test result.

12 **Alex:** That helps the [4] _____ assess the concern through the current plan, rather than relying on an informal team conclusion.

13 **Noor:** Who will explain any decision afterward so I do not hear different versions from different people?

14 **Alex:** We should identify the communication owner and ensure the athlete receives the verified outcome, not only the staff discussion.

15 **Noor:** If the plan changes, I want to know which earlier instruction is no longer current.

16 **Alex:** A [5] _____ should make that explicit and correct any conflicting message through the appropriate process.

17 **Noor:** Then I do not need to choose between sounding positive and telling the truth about the difficulty.

18 **Alex:** Exactly. We can acknowledge effort without asking you to minimize a concern or promise a recovery date.

19 **Noor:** I will give the factual account and follow the actual clinical instructions for reporting and response.

20 **Alex:** That supports [6] _____ while leaving diagnosis, urgency, and any plan change to the appropriate clinical process.

ANSWERS / EXPLANATIONS: PAGE 103

1. event description | 2. Performance pressure | 3. task difficulty | 4. responsible reviewer | 5. plan clarification | 6. nonjudgmental reporting

A missed entry concealed a concern

F Say it in a new situation

An athlete omitted a reported difficulty from a progress form because they feared blame. The team corrects the record and routes the actual concern for clinical review; no diagnosis is supplied.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

established / account / blame / incomplete

Athlete: "The progress entry was [1] _____."

Clinician: "We need the factual [2] _____."

Athlete: "A diagnosis is not yet [3] _____."

Clinician: "We can correct the record without assigning [4] _____."

ANSWERS / EXPLANATIONS: PAGE 103

1. incomplete | 2. account | 3. established | 4. blame

Read, switch, repeat

1. Check the four answers and read the complete exchange aloud.

2. Switch roles. Read it again with the word bank covered.

3. Return to the extended conversation. Read turns 1-10, then switch roles for turns 11-20. Use the corrected expressions.

SOLO PRACTICE

Read both roles. Pause briefly at each change of speaker. Repeat any line that needed an answer check.

01 Briefing and language checks

Clarifying an injury mechanism

A Briefing checks / page 4

1. Which mechanism is reported?

B. Planting the left foot and turning without contact
The brief specifies a left-foot plant and turn without another player's contact.

2. Which phrase needs clarification?

B. Gave way
Gave way could describe different experiences and has not been clarified in the history.

3. What is not yet supplied?

B. A confirmed diagnosis
The case contains a symptom history but no examination, imaging, or diagnosis.

D Language checks / page 9

1. Which opening question resolves the key ambiguity?

B. What happened when you say the knee gave way?
The question clarifies the athlete's own phrase without assuming a diagnosis or examination result.

2. Which summary preserves the reported mechanism?

C. Left foot planted; turn without player contact
Only the movement-based summary matches the supplied movement, side, and absence of contact.

3. Which distinction is anatomically appropriate?

D. A sprain involves a ligament; a strain involves muscle or tendon
The terms identify different tissue categories without diagnosing this athlete's injury.

4. Which claim exceeds the case?

B. The ligament is definitely torn
No structural diagnosis is supplied, so a definite tear would add unsupported information.

01 Conversation answers

Clarifying an injury mechanism

E Dialogue gaps 1-5

Pages 10-11

1. mechanism of injury. Turn 2. The mechanism of injury describes the event and movement, not a confirmed structural diagnosis.

2. buckling. Turn 4. Buckling describes a giving-way movement and should not be assumed when the athlete has not clarified what occurred.

3. pivot. Turn 6. A pivot is a turn around a planted or supporting foot, matching the movement described by the athlete.

4. weight-bearing. Turn 8. Weight-bearing concerns supporting body weight through the limb, which is a functional detail rather than a diagnosis.

5. swelling. Turn 10. Swelling describes an observed or reported increase in size; it does not by itself identify the injured tissue.

F Transfer answers 1-2

Page 12

1. right

Athlete: "The affected ankle was the right one."

The athlete explicitly corrects the side to the right ankle.

2. left

Clinician: "The earlier left entry needs correction."

The left-side entry is the incorrect earlier information being corrected.

E Dialogue gaps 6-10

Pages 10-11

6. prior injury. Turn 12. A prior injury is an earlier event that may be relevant to the current history without automatically being the same current condition.

7. aggravating factor. Turn 14. An aggravating factor is an activity associated with worsening symptoms, not proof of a particular tissue diagnosis.

8. ligament. Turn 16. A ligament links bones across a joint, but mentioning that structure does not establish that it is injured in this case.

9. strain. Turn 18. A strain concerns muscle or tendon, while a sprain concerns ligament tissue; neither is confirmed by this history alone.

10. non-contact injury. Turn 20. A non-contact injury description identifies the absence of direct contact in the event, not a conclusion that the injury is minor or harmless.

F Transfer answers 3-4

Page 12

3. unconfirmed

Athlete: "The diagnosis remains unconfirmed."

A corrected side does not itself establish a clinical diagnosis.

4. source

Clinician: "I will preserve the correction's source."

The record should identify where the clarification came from under the actual process.

02 Briefing and language checks

Explaining an examination and inviting a pause

A Briefing checks / page 13

1. What does Mo initially confuse?

A. Movement range and strength
The case explicitly describes confusion between range of motion and strength.

2. Which action remains available to Mo?

A. Ask to pause
The explanation makes clear that Mo can request a pause during the actual process.

3. What is not supplied?

D. A measured angle
No measured range, maneuver, or examination finding appears in the case.

D Language checks / page 18

1. Which explanation distinguishes active and passive correctly?

A. You move it yourself versus movement guided by another
The terms describe how movement is produced, not character, strength, or a diagnosis.

2. Which instruction supports ongoing consent?

D. Tell me if you want to pause before we continue
This statement keeps the person's ability to pause and clarify visible.

3. Which pair should not be treated as identical?

B. Range of motion and muscle strength
Movement extent and force production are different assessment concepts requiring clear language.

4. Which claim would invent a finding?

B. Your range is exactly normal
No measurement or finding is provided, so normal would be an unsupported result.

02 Conversation answers

Explaining an examination and inviting a pause

E Dialogue gaps 1-5

Pages 19-20

1. range of motion. Turn 2. Range of motion describes how far a joint moves, which differs from the force measured in a strength assessment.

2. baseline measurement. Turn 4. A baseline measurement provides an initial comparison point and is not automatically a complete diagnosis or a future clearance decision.

3. active movement. Turn 6. Active movement is performed by the person using their own muscles, rather than being guided entirely by another person.

4. passive movement. Turn 8. Passive movement is guided by another person or external force and does not mean the athlete is uncooperative.

5. palpation. Turn 10. Palpation is examination by touch and should be explained within the actual examination and consent process.

F Transfer answers 1-2

Page 21

1. pause

Athlete: "I would like to pause before the next step."

The athlete explicitly requests a pause to clarify the explanation.

2. explanation

Clinician: "Let us clarify the explanation first."

Clarification is needed before continuing the discussed examination step.

E Dialogue gaps 6-10

Pages 19-20

6. stop signal. Turn 12. A stop signal gives the person a clear way to request a pause or stop during the actual examination process.

7. examination consent. Turn 14. Examination consent concerns the explained examination and remains specific to the actual process rather than being assumed for every possible step.

8. chaperone. Turn 16. A chaperone is a person present during an examination under the relevant policy and preferences, which should be explained rather than assumed.

9. functional task. Turn 18. A functional task examines an activity relevant to everyday or sporting demands, which is different from one isolated movement measurement.

10. muscle strength. Turn 20. Muscle strength concerns force production, and distinguishing it from motion and function helps prevent an oversimplified interpretation of the examination.

F Transfer answers 3-4

Page 21

3. taken

Athlete: "No measurement has been taken."

The case states that no measurement has yet occurred.

4. finding

Clinician: "There is no established examination finding."

Without an examination result, the dialogue cannot supply an established finding.

03 Briefing and language checks

Reading back a rehabilitation plan

A Briefing checks / page 22

1. Which document governs the supplied communication?

C. The current signed rehabilitation plan
The current signed plan is the verified instruction identified in the case.

2. What must happen before progression?

A. The stated clinical review
The supplied plan explicitly requires a review before progression.

3. Which detail is absent?

D. Exercise dosage
No exercise, dosage, resistance, or progression criterion is provided here.

D Language checks / page 27

1. Which read-back matches the plan?

D. Supervised rehabilitation, with review before progression
The current plan specifies supervision and a review before progression.

2. Why is do more an inadequate instruction?

A. It leaves the dimension of change unclear
More does not identify whether load, frequency, repetitions, or duration should change.

3. Which action handles the conflicting documents appropriately?

A. Verify the current version with the responsible clinician
The discrepancy needs authorized clarification, not an improvised mixture of instructions.

4. Which claim exceeds this exercise?

A. The athlete should use a particular resistance
No resistance or exercise dosage is supplied, so a specific value would be invented.

03 Conversation answers

Reading back a rehabilitation plan

E Dialogue gaps 1-5

Pages 28-29

1. plan version. Turn 2. Plan version identifies which dated and authorized instruction is current when documents contain different wording.

2. rehabilitation plan. Turn 4. The rehabilitation plan is the current agreed clinical instruction, which must remain distinct from an older generic handout.

3. frequency. Turn 6. Frequency describes how often an activity occurs and cannot be inferred from the ambiguous phrase do more.

4. repetition. Turn 8. A repetition is one complete performance of a specified movement, whereas a set groups repetitions under the prescribed plan.

5. Progression. Turn 10. Progression changes the activity or demand under the appropriate clinical criteria and is not authorized by understanding the vocabulary.

F Transfer answers 1-2

Page 30

1. frequency

Athlete: "More often refers to frequency."

Frequency describes how often the specified activity occurs in the plan.

2. load

Clinician: "Greater resistance changes the opposing load."

Resistance concerns the load opposing an effort, not how often it occurs.

E Dialogue gaps 6-10

Pages 28-29

6. supervised session. Turn 12. A supervised session requires the specified professional oversight and should not be relabeled as an independent home activity.

7. symptom response. Turn 14. Symptom response describes what happened during or after the specified activity and should be reported without inventing a clinical interpretation.

8. prescribing clinician. Turn 16. The prescribing clinician is responsible for the relevant plan and needs accurate information when instructions or responses require review.

9. superseded instruction. Turn 18. A superseded instruction is an earlier direction replaced by the verified current plan and should not continue circulating as current guidance.

10. review criterion. Turn 20. A review criterion belongs to the actual clinical decision process and must not be replaced with a guessed timetable or an invented test.

F Transfer answers 3-4

Page 30

3. prescribed

Athlete: "No plan change has been prescribed."

The case explicitly provides no authorized change to the current plan.

4. clarification

Clinician: "The actual instruction needs appropriate clarification."

Unclear directions should be clarified through the responsible clinical process rather than guessed.

04 Briefing and language checks

Communicating concussion follow-up clearly

A Briefing checks / page 31

1. What does the current note address?

C. Specified classroom adjustments
The supplied note concerns school arrangements, not sport participation clearance.

2. Who must handle the clinical authorization question?

C. The responsible healthcare professional
Actual return-to-sport authorization belongs within the appropriate clinical process.

3. Which inference is unsupported?

B. Attending class means cleared for contact play
The classroom arrangement does not establish permission for contact practice or competition.

D Language checks / page 36

1. Which reply corrects the coach's inference?

D. Classroom arrangements do not authorize contact practice
Different settings and demands require their actual authorization; attendance alone is insufficient.

2. What detail should accompany cleared?

C. The activity and conditions authorized
Clearance must specify its scope rather than imply permission for every activity.

3. Which wording preserves the evidence limit?

C. No symptom score is supplied in this update
The exercise supplies no symptom score or finding, so the other statements invent reassurance.

4. What does the match date establish?

B. A scheduling pressure, not clinical authorization
A competition date is an organizational fact and cannot establish clinical clearance.

04 Conversation answers

Communicating concussion follow-up clearly

E Dialogue gaps 1-5

Pages 37-38

1. return to learn. Turn 2. Return to learn concerns supported learning; it does not authorize sporting contact or competition.

2. participation status. Turn 4. Participation status describes the current authorized activities and conditions rather than an unrestricted impression of recovery.

3. Physical exertion. Turn 6. Physical exertion involves bodily demands and cannot be assumed equivalent to the learning arrangements described in a school note.

4. Cognitive exertion. Turn 8. Cognitive exertion concerns mental effort, distinct from the physical demands of sports practice.

5. medical clearance. Turn 10. Medical clearance must come from the appropriate healthcare professional and specify the activity being authorized.

E Dialogue gaps 6-10

Pages 37-38

6. symptom inventory. Turn 12. A symptom inventory records reported symptoms; the classroom note supplies no such assessment.

7. staged progression. Turn 14. Staged progression follows the applicable clinical process and is not completed merely because a competition date is approaching.

8. Contact practice. Turn 16. Contact practice involves specific physical demands that must not be inferred as permitted from a classroom attendance arrangement.

9. clinical reassessment. Turn 18. Clinical reassessment means further professional evaluation under the actual clinical process.

10. Return to sport. Turn 20. Return to sport is a clinician-guided process whose authorization must be distinguished from the supplied return-to-learn arrangements.

F Transfer answers 1-2

Page 39

1. non-contact

Clinician: "The authorization is for the specified non-contact activity."

The current note explicitly limits the authorization to the specified non-contact activity.

2. competition

Coach: "It does not authorize competition."

Competition is not included in the authorization described in the brief.

F Transfer answers 3-4

Page 39

3. scope

Clinician: "The shorter message lost the scope."

The word cleared alone omitted the activity and limits of the authorization.

4. information

Coach: "I will send the accurate participation information."

The corrected message should preserve the actual permission rather than broaden it.

05 Briefing and language checks

Comparing training load without false equivalence

A Briefing checks / page 40

1. Which detail is the same in both records?

B. Duration of 30 minutes
Both sessions last thirty minutes, but their recorded demands differ.

2. What distinguishes the sessions?

C. Easy cycling versus repeated hard intervals
The logs distinguish session type and effort, without supplying clinical outcomes.

3. What is not provided by this comparison?

C. An individualized progression recommendation
The case compares language and records, not a personal training prescription.

D Language checks / page 45

1. Which comparison is justified by the logs?

A. Equal duration but different recorded intensity
The logs show matching minutes and different activity demands, not equivalent overall effects.

2. What does perceived exertion identify?

D. The person's reported effort
Perceived exertion is the person's experience of effort, interpreted using the stated method.

3. Why clarify the weekly date range?

B. Totals need matching time windows
Different included dates can make apparently similar weekly totals refer to different periods.

4. Which statement overreaches?

A. This wearable number guarantees no injury
A device metric alone cannot guarantee safety or predict the absence of injury.

05 Conversation answers

Comparing training load without false equivalence

E Dialogue gaps 1-5

Pages 46-47

1. training load. Turn 2. Training load includes the amount and character of the demand, not simply the number of minutes spent exercising.

2. intensity. Turn 4. Intensity describes the degree of effort or demand, which can differ substantially between sessions with equal duration.

3. session RPE. Turn 6. Session RPE is a rating of perceived exertion under a defined method, so the scale and recording approach need to be known.

4. external load. Turn 8. External load concerns the work performed, such as measured power or distance, rather than the individual's response to that work.

5. Internal load. Turn 10. Internal load concerns the person's physiological or perceived response and should be interpreted in context rather than treated as a diagnosis.

F Transfer answers 1-2

Page 48

1. equal

Athlete: "The distance totals are equal."

Both records show twenty kilometers, so the stated distance totals match.

2. different

Clinician: "The recording windows are different."

One record ends Friday while the other includes the whole week.

E Dialogue gaps 6-10

Pages 46-47

6. work-to-rest ratio. Turn 12. The work-to-rest ratio describes the relationship between effort and recovery periods, helping distinguish sessions that share a broad label.

7. weekly mileage. Turn 14. Weekly mileage must identify the included dates so distance totals from different recording windows are not compared as identical periods.

8. wearable metric. Turn 16. A wearable metric is a device measurement or estimate and does not automatically provide clinical clearance or an individualized training decision.

9. monitoring limitation. Turn 18. A monitoring limitation explains what the available data cannot establish, including a guaranteed injury prediction or safety conclusion.

10. recovery status. Turn 20. Recovery status requires the relevant actual assessment and cannot be settled by matching session durations or relying on one device estimate.

F Transfer answers 3-4

Page 48

3. absent

Athlete: "Intensity information is absent."

The brief supplies no intensity data for either activity record.

4. demand

Clinician: "Equal distance does not establish equivalent demand."

Different time windows and missing intensity prevent an overall equivalence conclusion.

06 Briefing and language checks

Explaining imaging findings in clinical context

A Briefing checks / page 49

1. What does the report request?

D. Clinical correlation
The report asks for interpretation alongside the actual clinical information.

2. Which detail is deliberately not supplied?

A. A named structural diagnosis
The case gives no specific structure, severity, or diagnosis to interpret.

3. What cannot be concluded from this exercise?

A. The athlete's sport clearance
Clearance requires the actual assessment and decision, neither of which is supplied here.

D Language checks / page 54

1. Which explanation best fits clinical correlation?

C. Interpret the finding with the history and examination
Clinical correlation explicitly connects the report with relevant clinical information.

2. Which conclusion does incidental not automatically support?

D. The finding is harmless and needs no review
Incidental describes discovery context and does not by itself settle clinical importance.

3. Which pair should remain distinct?

D. Structural finding and functional limitation
A structural description and an activity limitation answer different questions in the assessment.

4. Which response handles uncertainty accurately?

B. This possible explanation needs clinical review
This response preserves the report's limits without dismissing imaging altogether.

06 Conversation answers

Explaining imaging findings in clinical context

E Dialogue gaps 1-5

Pages 55-56

1. Clinical correlation. Turn 2. Clinical correlation means interpreting the finding alongside the history, examination, and other relevant information rather than reading the image in isolation.

2. structural finding. Turn 4. A structural finding describes anatomy and does not automatically establish the cause of every reported symptom.

3. report impression. Turn 6. The report impression summarizes the imaging interpretation, but it remains part of a report that must be read in clinical context.

4. incidental finding. Turn 8. An incidental finding was discovered outside the examination's primary question, which does not automatically establish that it is clinically unimportant.

5. functional limitation. Turn 10. A functional limitation concerns an activity the person has difficulty performing, which is distinct from the structural description on an image.

F Transfer answers 1-2

Page 57

1. suggests

Clinician: "The original report said suggests."

Suggests preserves a possible interpretation rather than asserting confirmation.

2. confirms

Colleague: "The summary incorrectly changed that to confirms."

Confirms increases certainty beyond the original report's stated meaning.

E Dialogue gaps 6-10

Pages 55-56

6. shared interpretation. Turn 12. A shared interpretation brings the relevant findings and perspectives together rather than letting one isolated report phrase determine every conclusion.

7. imaging limitation. Turn 14. An imaging limitation identifies what the examination cannot reliably show or establish and belongs in a careful interpretation of the findings.

8. artifact. Turn 16. An artifact can arise from technical or acquisition factors and should not be assumed to represent the anatomical finding someone expects.

9. clinical significance. Turn 18. Clinical significance concerns what the finding means for the actual person and clinical question, not merely how dramatic the wording sounds.

10. prognosis. Turn 20. A prognosis describes an expected course with appropriate uncertainty and cannot be guaranteed from a single isolated imaging phrase.

F Transfer answers 3-4

Page 57

3. remains

Clinician: "The need for clinical correlation remains."

No additional assessment is supplied to resolve the original requirement for context.

4. certainty

Colleague: "I will correct the overstated certainty."

The correction must restore the original level of certainty instead of repeating an unsupported conclusion.

07 Briefing and language checks

Discussing return-to-sport criteria under pressure

A Briefing checks / page 58

1. What is the current competition status?

A. Not yet authorized
The brief explicitly states that competition has not been authorized.

2. What is pending?

D. Review of relevant criteria
The actual review remains pending, with no criteria results provided here.

3. What does two weeks identify?

C. Time until the final
Two weeks is the event timing and cannot be treated as a clinical recovery guarantee.

D Language checks / page 63

1. Which sentence preserves the condition?

D. If the actual review supports authorization, the team will communicate its scope
If keeps the outcome conditional on the actual review rather than promising clearance.

2. Which distinction helps answer the coach?

C. Clinical authorization and team selection
Selection is an organizational decision, while clinical authorization has its own responsible process.

3. Which reply acknowledges pressure without promising recovery?

B. I understand the final matters; the review still determines the decision
The reply recognizes the athlete's goal while preserving the unresolved clinical decision.

4. What is not supplied in this exercise?

B. A universal pass score
No test result or clearance standard is given, so the learner must not invent one.

07 Conversation answers

Discussing return-to-sport criteria under pressure

E Dialogue gaps 1-5

Pages 64-65

1. selection pressure. Turn 2. Selection pressure concerns the athlete's role or opportunity in the team and should be acknowledged without changing the clinical decision process.

2. time-based milestone. Turn 4. A time-based milestone identifies a point on the calendar and does not establish that clinical or functional criteria have been satisfied.

3. Return to participation. Turn 6. Return to participation can involve limited sporting activity under defined conditions and differs from full competition or previous performance.

4. conditional authorization. Turn 8. Conditional authorization must preserve its stated limits and conditions rather than becoming an unrestricted permission when summarized.

5. Functional testing. Turn 10. Functional testing assesses relevant task performance within the actual clinical process, and this scenario supplies no completed results to report.

F Transfer answers 1-2

Page 66

1. supervision

Clinician: "The note requires supervision."

The actual authorization explicitly includes supervision as a condition.

2. unrestricted

Coach: "The summary incorrectly said unrestricted training."

Unrestricted broadens the permission beyond the limited activity in the note.

E Dialogue gaps 6-10

Pages 64-65

6. sport-specific demand. Turn 12. A sport-specific demand concerns a task particular to the sport or role and can differ from the demands of everyday movement.

7. Psychological readiness. Turn 14. Psychological readiness concerns mental preparedness for return and should be discussed respectfully rather than used as a character judgment.

8. review outcome. Turn 16. The review outcome is the result of the actual assessment process, which remains pending in the supplied case.

9. decision rationale. Turn 18. The decision rationale states the basis for the clinical position rather than relying on team pressure or an event date.

10. criteria-based decision. Turn 20. A criteria-based decision depends on the relevant actual evidence and conditions rather than elapsed time or competitive pressure alone.

F Transfer answers 3-4

Page 66

3. condition

Clinician: "That change removed a condition."

The shorter message omitted a condition that limits the authorization.

4. scope

Coach: "I will restore the actual scope."

The corrected summary must preserve the activity and conditions actually authorized.

08 Briefing and language checks

Sharing participation information without oversharing

A Briefing checks / page 67

1. What does the coordinator need for the stated task?

C. Authorized participation information
The task is practice planning, for which the relevant participation information is needed.

2. What permission is documented?

A. A limited participation update
The brief explicitly limits the documented permission to a participation update.

3. What must govern access to health details?

C. Relevant consent, law, and institutional policy
Actual information access depends on the applicable requirements, not a universal team entitlement.

D Language checks / page 72

1. Which reply is both useful and appropriately bounded?

C. Let us confirm the authorized participation update for practice planning
This reply addresses the legitimate task while preserving the actual disclosure limits.

2. What does documented limited permission not establish?

C. Unrestricted chart access
Limited permission cannot be expanded into blanket access to all health information.

3. Which question should precede forwarding?

D. Is the new recipient and purpose authorized?
Changing the recipient or purpose can change whether the actual disclosure is authorized.

4. Which distinction protects professional judgment?

C. Clinical authorization and team selection pressure
Clinical decisions must retain the appropriate professional authority rather than be dictated by selection pressure.

08 Conversation answers

Sharing participation information without oversharing

E Dialogue gaps 1-5

Pages 73-74

1. disclosure purpose. Turn 2. Disclosure purpose identifies why information is requested and helps determine which authorized information is relevant to the task.

2. status update. Turn 4. A status update communicates the current relevant position for the task and need not include every detail in the clinical chart.

3. release authorization. Turn 6. Release authorization concerns specified information sharing and cannot be treated as blanket permission for unrestricted chart distribution.

4. recipient. Turn 8. A recipient is the person or organization receiving information, and their role and authorization matter when the message is shared.

5. data minimization. Turn 10. Data minimization limits information to what is appropriate for the legitimate purpose and applicable requirements rather than sharing everything available.

F Transfer answers 1-2

Page 75

1. liaison

Coordinator: "The authorized recipient is the team liaison."

The permission in the brief names the team liaison, not every outside party.

2. recipient

Clinician: "The sponsor is a different recipient."

Adding an external sponsor changes who would receive the health-related information.

E Dialogue gaps 6-10

Pages 73-74

6. secure channel. Turn 12. A secure channel is an authorized communication route with appropriate protections, not merely the most convenient available messaging option.

7. professional independence. Turn 14. Professional independence preserves the appropriate clinical judgment rather than allowing team selection pressure to determine the medical position.

8. Role-based access. Turn 16. Role-based access ties information permissions to authorized work roles and does not grant every team member identical access to health records.

9. audit trail. Turn 18. An audit trail records relevant access, sharing, or changes so the handling of information can be traced under the actual process.

10. Confidentiality. Turn 20. Confidentiality protects information under the relevant professional and legal requirements while allowing appropriately authorized communication for legitimate purposes.

F Transfer answers 3-4

Page 75

3. approved

Coordinator: "Sponsor disclosure has not been approved."

The case explicitly states that no sponsor disclosure has been approved.

4. scope

Clinician: "We must check the actual authorization scope."

The authorization's limits must be checked before information is shared more broadly.

09 Conversation answers

Clarifying a therapeutic use exemption request

E Extended conversation

Pages 76-77

1. therapeutic use exemption. Turn 4. A therapeutic use exemption concerns permission under the applicable anti-doping process and is not automatically established by a prescription alone.

2. Application receipt. Turn 8. Application receipt confirms that a request arrived and does not itself establish approval of the exemption being requested.

3. athlete classification. Turn 10. Athlete classification can affect the relevant anti-doping process and responsible organization, so it must be established rather than guessed.

4. pending decision. Turn 14. A pending decision means the relevant determination has not been completed and must not be presented as a favorable outcome.

5. responsible anti-doping organization. Turn 18. The responsible anti-doping organization handles the applicable sporting process, which must be identified from the actual athlete and rule context.

6. authorization distinction. Turn 20. The authorization distinction separates the clinical prescription from permission under sporting rules, preventing one decision from being treated as the other.

F Transfer exchange

Page 78

1. received

Athlete: "The application has been received."

The portal confirms arrival of the application, not its approval.

2. requested

Clinician: "Further information is still requested."

The portal asks for additional information before the matter is resolved.

3. given

Athlete: "Approval has not been given."

No approval decision appears in the supplied status update.

4. status

Clinician: "We should report the actual application status."

Accurate status language prevents a receipt notice from becoming a false authorization.

10 Conversation answers

Handing over care before overseas travel

E Extended conversation

Pages 79-80

1. document provenance. Turn 4. Document provenance identifies the origin and version of the plan, helping the receiving clinician distinguish current instructions from older copies.

2. travel purpose. Turn 6. Travel purpose explains why the person is attending the event and does not establish clinical authorization to participate in every activity there.

3. handoff responsibility. Turn 8. Handoff responsibility identifies who owns each part of the ongoing care and communication rather than assuming a sent document transfers every duty.

4. time zone. Turn 10. A time zone makes a cross-border contact time interpretable and prevents identical clock numbers from referring to different moments.

5. booking status. Turn 14. Booking status distinguishes a requested appointment from an accepted or confirmed booking and should be communicated accurately before travel.

6. continuity of care. Turn 18. Continuity of care depends on connected information and responsibilities across services, not simply on sending a complete-looking document before travel.

F Transfer exchange

Page 81

1. zone

Referrer: "The call still needs a date and time zone."

The clock time alone is ambiguous when the teams work across locations.

2. specified

Receiver: "The proposed contact time is not fully specified."

The missing date and zone prevent a complete shared understanding of the appointment.

3. unchanged

Referrer: "The clinical plan remains unchanged."

Clarifying a contact time does not alter any clinical instruction.

4. details

Receiver: "We will confirm the complete call details."

A usable confirmation must include the missing information needed by both teams.

11 Conversation answers

Discussing a return setback without assigning blame

E Extended conversation

Pages 82-83

1. event description. Turn 4. An event description records what occurred without automatically assigning a diagnosis, cause, or change to the rehabilitation plan.

2. Performance pressure. Turn 6. Performance pressure can influence how an athlete reports concerns, but it should not replace accurate communication or the clinical response.

3. task difficulty. Turn 8. Task difficulty describes a problem performing the activity and does not by itself establish treatment failure or a new structural injury.

4. responsible reviewer. Turn 12. The responsible reviewer needs accurate information to assess the concern and should be identified rather than replaced by an informal team opinion.

5. plan clarification. Turn 16. A plan clarification makes the actual instruction and any authorized change understandable without asking the athlete to resolve conflicting versions alone.

6. nonjudgmental reporting. Turn 20. Nonjudgmental reporting describes relevant facts without attributing blame or motive, helping the clinical team receive information needed for an appropriate response.

F Transfer exchange

Page 84

1. incomplete

Athlete: "The progress entry was incomplete."

Omitting the reported difficulty left the progress record without relevant information.

2. account

Clinician: "We need the factual account."

The review needs what actually happened rather than an invented interpretation.

3. established

Athlete: "A diagnosis is not yet established."

The scenario provides a report and review route but no clinical diagnosis.

4. blame

Clinician: "We can correct the record without assigning blame."

A nonjudgmental correction preserves the facts without attributing an unsupported negative motive.

Find the move you need

Use these as precise language models. Replace details only when you know the actual facts, authority, and agreed conditions.

01 Clarifying an injury mechanism

Locate the event: Which foot was on the ground when you turned?

Separate sequence: Did the discomfort begin before, during, or after the turn?

Avoid a shortcut: We should not replace your description with a diagnosis yet.

Full phrase set: pages 7-8 | Dialogue: pages 10-11

02 Explaining an examination and inviting a pause

Explain the purpose: This part looks at how the shoulder moves, not only how strong it is.

Invite a pause: Please tell me if you want to pause or stop.

Keep consent specific: We can discuss this step separately before proceeding.

Full phrase set: pages 16-17 | Dialogue: pages 19-20

03 Reading back a rehabilitation plan

Check the version: Which dated plan are we reading together?

Read back: My understanding is supervised rehabilitation, then review before progression.

Confirm the limit: Completing this conversation does not change the clinical plan.

Full phrase set: pages 25-26 | Dialogue: pages 28-29

04 Communicating concussion follow-up clearly

Separate the permissions: The school note addresses classroom arrangements, not contact practice.

Identify the decision-maker: The responsible healthcare professional must clarify that authorization.

Confirm receipt: Please read back the participation limit your team has received.

Full phrase set: pages 34-35 | Dialogue: pages 37-38

Keep the conversation productive

Use these as precise language models. Replace details only when you know the actual facts, authority, and agreed conditions.

05 Comparing training load without false equivalence

Align the measure: Are we comparing time, distance, power, or reported effort?

Define the window: What dates are included in the weekly total?

Summarize precisely: Equal time does not mean an equivalent training session.

Full phrase set: pages 43-44 | Dialogue: pages 46-47

06 Explaining imaging findings in clinical context

Explain correlation: We need to connect this finding with the history and examination.

Keep limits visible: The examination has limits that the report should describe.

Separate permission: An imaging phrase is not itself sport clearance.

Full phrase set: pages 52-53 | Dialogue: pages 55-56

07 Discussing return-to-sport criteria under pressure

Acknowledge the goal: I understand why this final matters to you.

Explain conditions: The decision depends on the actual review and relevant findings.

Close with the next step: We will communicate the actual review outcome and its scope.

Full phrase set: pages 61-62 | Dialogue: pages 64-65

08 Sharing participation information without oversharing

Clarify the task: What decision do you need this information to support?

Check the recipient: Who will receive the message, and will it be forwarded?

Close constructively: I will clarify what can be shared for this specific purpose.

Full phrase set: pages 70-71 | Dialogue: pages 73-74

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

abduction explain abduction 14

academic adjustment confirm an academic adjustment 32

action owner confirm the action owner 6

action owner confirm the action owner 15

action owner confirm the action owner 24

action owner confirm the action owner 33

action owner confirm the action owner 42

action owner confirm the action owner 51

action owner confirm the action owner 60

action owner confirm the action owner 69

active movement explain active movement 14

acute workload define the acute workload window 42

adduction explain adduction 14

aggravating factor identify an aggravating factor 6

artifact identify a possible artifact 51

athlete representation clarify athlete representation 68

audit trail retain an audit trail 69

authorized disclosure confirm authorized disclosure 68

baseline measurement record a baseline measurement 15

buckling describe an episode of buckling 5

capacity describe assessed capacity 60

chaperone offer a chaperone appropriately 15

check-back complete a check-back 6

check-back complete a check-back 15

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

check-back complete a check-back 24
check-back complete a check-back 33
check-back complete a check-back 42
check-back complete a check-back 51
check-back complete a check-back 60
check-back complete a check-back 69
chronic workload define the chronic workload window 42
clarification request make a targeted clarification request 6
clarification request make a targeted clarification request 15
clarification request make a targeted clarification request 24
clarification request make a targeted clarification request 33
clarification request make a targeted clarification request 42
clarification request make a targeted clarification request 51
clarification request make a targeted clarification request 60
clarification request make a targeted clarification request 69
clinical correlation seek clinical correlation 50
clinical reassessment arrange clinical reassessment 33
clinical record protect the clinical record 68
clinical significance clarify clinical significance 50
cognitive exertion discuss cognitive exertion 32
comparison study identify the comparison study 50
competition readiness discuss competition readiness 59
concentration difficulty describe concentration difficulty 32
concussion communicate a concussion concern 32

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

conditional authorization communicate conditional authorization 60
confidence in movement explore confidence in movement 59
confidentiality maintain confidentiality 68
conflict of interest disclose a conflict of interest 69
contact practice distinguish contact practice 32
criteria-based decision communicate a criteria-based decision 59
data minimization apply data minimization 68
decision rationale explain the decision rationale 60
diagnostic ultrasound explain diagnostic ultrasound 50
differential diagnosis explain a differential diagnosis 51
disclosure purpose specify the disclosure purpose 68
dizziness describe dizziness 32
duration distinguish duration from frequency 23
effusion assess a joint effusion 5
examination consent confirm examination consent 15
exercise adherence discuss exercise adherence accurately 24
exercise prescription verify an exercise prescription 23
extension describe extension 14
external load quantify external load 41
external rotation describe external rotation 14
fatigue report fatigue in context 42
fear of reinjury acknowledge fear of reinjury 59
flexion explain shoulder flexion in context 14
frequency clarify exercise frequency 23

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

functional limitation describe a functional limitation 51
functional task clarify a functional task 15
functional testing explain functional testing 59
graded exposure clarify graded exposure 59
headache report a headache accurately 32
home program review a home program 23
image quality describe image quality 50
imaging limitation state an imaging limitation 50
imaging report review the imaging report 50
incidental finding explain an incidental finding 50
information governance follow information governance 69
intensity distinguish training intensity 41
internal load describe internal load 41
internal rotation describe internal rotation 14
interval training describe interval training 41
joint instability distinguish reported joint instability 5
ligament explain ligament anatomy 5
load tolerance assess load tolerance 23
locking clarify reported locking 5
magnetic resonance imaging discuss magnetic resonance imaging 50
mechanism of injury clarify the mechanism of injury 5
medical clearance verify medical clearance 33
meniscus explain the meniscus 5
monitoring limitation state a monitoring limitation 42

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

movement quality assess movement quality 59
muscle strength assess muscle strength 15
need-to-know access apply need-to-know access 68
non-contact activity specify non-contact activity 33
non-contact injury describe a non-contact injury 5
palpation explain palpation before contact 14
participation restriction communicate a participation restriction 68
participation status communicate participation status 33
passive movement explain passive movement 14
perceived exertion clarify perceived exertion 41
performance demand specify the performance demand 60
photophobia explain photophobia 32
physical exertion distinguish physical exertion 32
pivot describe a pivot 5
plan version confirm the plan version 24
prescribing clinician identify the prescribing clinician 24
prior injury clarify a prior injury 6
professional independence preserve professional independence 69
prognosis discuss prognosis cautiously 51
progression authorize a progression 23
psychological readiness discuss psychological readiness 59
radiograph explain a radiograph 50
range of motion assess range of motion 14
recipient identify the intended recipient 68

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

recovery interval specify the recovery interval 41
recovery status discuss recovery status 42
regression clarify a regression 23
rehabilitation plan clarify the rehabilitation plan 23
release authorization verify a release authorization 68
relieving factor identify a relieving factor 6
repetition distinguish a repetition 23
report impression explain the report impression 50
resistance specify the actual resistance 23
resisted movement describe resisted movement 14
return to learn clarify return-to-learn arrangements 32
return to participation distinguish return to participation 59
return to performance distinguish return to performance 59
return to sport clarify return-to-sport status 32
review criterion clarify a review criterion 24
review outcome communicate the review outcome 60
role-based access check role-based access 68
scope boundary state the scope boundary 6
scope boundary state the scope boundary 15
scope boundary state the scope boundary 24
scope boundary state the scope boundary 33
scope boundary state the scope boundary 42
scope boundary state the scope boundary 51
scope boundary state the scope boundary 60

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

scope boundary state the scope boundary 69
secure channel use a secure channel 68
selection pressure acknowledge selection pressure 60
session RPE record session RPE 41
set distinguish a set 23
shared interpretation support shared interpretation 51
source attribution preserve source attribution 6
source attribution preserve source attribution 15
source attribution preserve source attribution 24
source attribution preserve source attribution 33
source attribution preserve source attribution 42
source attribution preserve source attribution 51
source attribution preserve source attribution 60
source attribution preserve source attribution 69
sport-specific demand identify sport-specific demands 59
sprain distinguish a sprain 6
staged progression follow the authorized staged progression 33
status update provide an accurate status update 69
steady-state activity compare steady-state activity 41
stop signal establish a stop signal 15
strain distinguish a strain 6
structural finding interpret a structural finding 50
superseded instruction identify a superseded instruction 24
supervised session confirm a supervised session 23

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

swelling describe the onset of swelling 5
symptom generator assess a possible symptom generator 51
symptom inventory review a symptom inventory 32
symptom onset establish symptom onset 6
symptom recurrence report symptom recurrence 33
symptom response report the symptom response 24
team liaison identify the team liaison 69
tenderness document assessed tenderness 14
tendon distinguish a tendon from a ligament 5
time-based milestone distinguish a time-based milestone 59
training load describe training load 41
training volume compare training volume 41
wearable metric interpret a wearable metric 42
weekly mileage compare weekly mileage 41
weight-bearing clarify weight-bearing ability 5
work-to-rest ratio clarify the work-to-rest ratio 41
working assumption challenge a working assumption 6
working assumption challenge a working assumption 15
working assumption challenge a working assumption 24
working assumption challenge a working assumption 33
working assumption challenge a working assumption 42
working assumption challenge a working assumption 51
working assumption challenge a working assumption 60
working assumption challenge a working assumption 69

Original cases. Clear boundaries.

These are original fictional teaching conversations, not transcripts of real workers. Names, figures, organizations, and incidents are invented. Sources support terminology and communication aims, not the fictional results or an endorsement of this book.

[1] AAOS OrthoInfo. Sprains, Strains, and Other Soft-Tissue Injuries.

Open the original source

Background for distinguishing tissue terminology and reported injury mechanisms; not a treatment plan. Checked 11 October 2026.

[2] CDC HEADS UP. Returning to Sports.

Open the original source

Supports the need for healthcare approval and supervision in concussion return-to-sport communication; the book does not reproduce a progression protocol. Checked 11 October 2026.

[3] AAOS OrthoInfo. Recovery and Rehabilitation.

Open the original source

Context for rehabilitation terminology and the need for instructions appropriate to the actual assessed injury. Checked 11 October 2026.

[4] US Anti-Doping Agency. Therapeutic Use Exemptions.

Open the original source

Context for distinguishing an exemption request from approval under the actual anti-doping process. Checked 11 October 2026.

Phil Stilwell

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